MANAGEMENT CONCEPTS

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FOR THE REGISTERED DIETITIAN NUTRITIONIST EXAMINATION

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Part 1

Management Overview

For the management section of the RD exam, don't be overwhelmed by the terms. Step one of being ready for this part of the test is being familiar with these terms and their definitions. Note that I didn't say "knowing the definitions", I said "be familiar with". You don't need to be able to write an essay about systems theory, but you should be able to recognize a definition referring to inputs, transformation, and outputs, as a key part of systems theory. Before we jump into the 5 Management Functions, let's do an overview. This overview isn't something to skim over – many of these overview ideas are tested on the exam. Plus, this overview provides context.

First, it's important to understand that systems theory and systems approach, functional organization, and management functions have similar concepts, but are not directly connected. They are distinct theories that are used together in practice.

The systems approach uses functional subsystems and the transformation of inputs to outputs. While the management functions are related to the systems approach, they are not directly involved in transformation and they are not linking processes between communications and decision making.

The ideas are separate but similar.

Systems Approach and Functional Organization	Management Functions
• Functional subsystems • Linking processes • Inputs, outputs, and transformation	• Planning, organization, staffing, directing, controlling • Used to coordinate subsystems

Procurement, production, distribution and service, sanitation, and maintenance are the **functional subsystems** within the foodservice system, each classified by function. Come back to this overview when we talk about **functional organization** and **systems approach** in Part II. Don't worry, I put a reminder in Part II for you.

The 5 management functions are planning, organization, staffing, directing, and controlling.[1] Planning and other management functions set plans and objectives, but they don't outline exactly **how** the objectives should be achieved. Furthermore, the 5 management functions don't change between departments or between subsystems. Managers in each department apply the same 5 management functions – **meaning that the 5 management functions are pervasive.** As you go through the details of each management function, build a framework in your mind on which to organize and remember the different definitions. Instead of just brute-force memorizing terms and definitions on flashcards, having context around the terms will help distinguish similar terms and definitions from one another.

Summarize the previous paragraph.

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The 3 skills you need to be a successful manager are:[1]

1. **Technical** – knowledge and capabilities to perform specific tasks
2. **Human/Interpersonal** – soft skills that should be nurtured, such as communication and attention to relationships (in contrast to hard skills that can be learned)
3. **Conceptual** – ability to integrate activities and interests of an organization → *the most important skill to have at higher levels of management*

There are **3 types of resources** core to all management principles.[2]

- Human resources
- Physical resources
- Financial resources

Write out and define each of the 3 skills necessary to be a successful manager. Which is the most important at higher levels of management?
What 3 types of resources are central to all management principles?

The 5 Management Functions

To begin, review the 5 management functions in this section. For applied management questions on the exam, like situational questions, you'll need to be able to identify examples of each function. Keeping this framework in mind will also help you learn the other concepts.

Management is the integration and coordination of resources to achieve the desired objectives of the organization. There are numerous functions, processes, methods, and theories on how to "do management".

The 5 management functions are used to coordinate subsystems in order to achieve objectives. Managing subsystems through the 5 functions is expected to result in the achievement of objectives.[1]

Planning – using problem solving and decision making to ensure proper utilization of all resources

Organization – assigning duties; delegating authority; creating responsibility

Staffing- recruiting and screening potential employees; hiring, orienting, and training new employees; supervising and evaluating employees

Directing – using leadership and management styles; using motivation and communication

Controlling – establishing standards for performance; evaluating actual performance; implementing corrective action

Can you think of a mnemonic to remember the 5 functions? *P O S D C*
Take out a few sheets of paper and label each one with the name of one of the 5 management functions. As you read through the next sections, write down what you think is important to know about each one.

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The 5 Management Functions

Planning

- The primary function of management. All other functions are performance within the framework of plans laid out.
- Planning is continuous, designed for efficiency, occurs through all level of management, and flexible.

Organization

- Organizing activities, assigning authority and responsibility, delegating
- Prioritize time and personnel resources
- Important because leads to well-defined jobs, clear lines of authority, coordination of roles, specialization of jobs, effective administration, growth and diversification, and scope for new changes

Staffing

- Recruiting and screening potential employees
- Hiring, orienting, and training new employees
- Supervising and evaluating employees

Directing

- Managers instruct, guide, and oversee the performance of workers to achieve goals
- Provides importance to planning, organization, and staffing
- Where the work actually starts to get done
- Involves the human/interpersonal component of management

Controlling

- Measure performance and take corrective action to ensure accomplishment of an organizational goal
- Ensures effective and efficient utilization of organizational resources

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Planning

The planning function of management involves looking ahead and preparing for the other management roles. Managers need to see the organization as it could be, not as it is. Planning is the primary function of management and the most basic. It involves formulating plans that balance needs and demands with the available resources (financial, physical, human).

Planning includes:
- Planning by objective
- Facilitating coordination and controlling
- Minimizing uncertainty

As a function, planning is:
- Goal oriented
- A continuous process
- Pervasive

Planning occurs at all levels of management and in all departments.

One key resource important in planning is **human resources planning** which ensures that an organization's (or department's) labor requirements are met.[1] Human resources planning includes forecasting staffing needs (FTEs) and analyzing labor market conditions.

Organization

Organization is the management function of grouping activities, delegating authority, and coordinating relationships (both horizontally and vertically). [1]

Organization includes:

- Identifying activities to complete and to be delegated, and grouping them
- Assigning **responsibility** for effective management – everyone should know who they are **accountable** to and what **authority** they have
- Successful planning and organization to avoid duplication of efforts which is a waste of resources (e.g. time, money, staff)

Organization is also key to maintain a safe work environment which is important to:

- Reduce use of workman's compensation, resulting in decreased costs
- Reduce lost workdays, resulting in decreased costs
- Demonstrate care for employees

Staffing

Staffing includes recruiting, screening, and selecting potential employees; hiring, orienting, and training new employees, and supervising and evaluating current employees. [1]

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Hiring

What is the difference between recruitment and selection? [1]

Recruitment is locating and encouraging potential applicants to apply for a job opening.

Selection is comparing applicant knowledge, skills, and abilities to those required of a position and choosing the most qualified applicant.

In hiring, the manager is responsible for writing job descriptions and job specifications. No matter the level of the job, a job description and specification must be written.

What is the difference between a job description and job specification?[1]

A **job description** includes job title, tasks, duties, roles, and responsibilities for a certain job – in sum, what the candidate can expect to do if they get the job.

A **job specification** includes the desired qualifications, skills, and abilities needed to complete the job – in sum, the job specification characterizes the *ideal* candidate.

On the day a new employee is hired, they should be given performance objectives.

Employee Training

The manager is also responsible for making sure there is a **planned new employee orientation** and **training program** in place.[1] Using a standardized program for all new hires means that all employees will be familiar with policies, procedures, job tasks, expectations, and other important aspects of the job. It also reduces the chance of miscommunication, like an employee saying they didn't know they needed to do something.

Managers should also have planned **development**, which is the process of improving the technical, human, and conceptual skills of employees. [1] An example would be attendance at webinars, lunch and learns, or in-service trainings.

In foodservice, managers should have planned **in-service trainings** that utilize the principles of adult learning and effective task teaching. To decide on an in-service training topic, a manager should observe staff and their work area to determine what topics need to be addressed, such as things to improve, things to work on, and things that may need to be explained more thoroughly.

How should a manager decide on an in-service training topic?

a. Choose from the job specifications
b. Observe staff and their work area
c. Ask a manager in another department
d. Find ideas online

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Employee Performance

Personnel policies should reflect the current realities of the specific work place. Policies should be kept up-to-date, and should be reinforced by strong supervision to make sure employees are being treated fairly and expectations are consistent.[3]

If an employee finds job tasks too demanding, how could you, as a manager, address the situation? You should help the employee improve their performance by empowering them and motivating them to do well.[1] Specifically, you can simplify their tasks, lessen their workload, and reduce time pressure, while maintaining a positive attitude and constructive conversation. You wouldn't re-design a job description to match their needs and to keep them in their current position. Having a conversation that re-iterates the performance standards for their job description also wouldn't be effective – it wouldn't actually reduce the job demands they feel or improve their performance. Providing additional resources, like staff assistance, modified equipment, and raw materials don't empower the employees.

Write out the scenario from the paragraph above and the options to address it. Explain why each possible option is ideal or not.

Employee Discipline

Consistency is key when disciplining employees – an employee should feel like they are getting the same treatment that any other employee would for the same offense. [1]

Identifying solutions to correct undesirable behaviors, providing feedback on performance, and identifying causes for unacceptable behavior are all processes involved in employee discipline specific to improving outcomes, but the most important, overarching idea is consistency.

What are 3 processes key to improving outcomes via employee discipline?

Progressive discipline is a graduated range of responses to employee performance problems or conduct problems. [1] The idea is that disciplinary responses should be appropriate to the problem. Employees should know what to expect, and employers are held to fair, consistent, and objective standards for discipline and proper documentation of both actions and decisions.

You may get questions on the RD exam that ask which response is most appropriate for a given situation. First consider what the problem in the scenario is – is it an employee's work quality? tardiness? poor conduct? Then, evaluate each answer choice in terms of addressing the issue, appropriateness of the response, and formal documentation. Someone being tardy for the first time, for example, doesn't warrant reporting that employee to the manager's manager.

Progressive discipline (escalation) includes these 4 steps for formal documentation: 1) verbal warning, 2) written warning, 3) suspension, 4) termination. The verbal warning itself would be documented, and if there are situations leading up to the verbal warning, those can be documented as well. For example, a manager might document unkempt employee appearance, poor availability when on call, or inappropriate conduct. These all serve as rationale for

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discipline. Note that informal conversations, like work-related advice from a supervisor, don't require documentation, especially documentation related to discipline or the employees "file". [1]

Directing

Directing involves the actual action steps taken from planning, organization, and staffing. Organizing determines which tasks should be delegated while directing actually delegates the tasks. Key skills for directing are **soft skills** (interpersonal skills).[2] Problem solving and communication are particularly important in directing.

Have you heard of "emotional intelligence"? Emotional intelligence includes a few skills, including **emotional awareness, emotional self-regulation, communication**, and **empathy**. [4] Emotional awareness is the ability to identify and name your own emotions. Emotional regulation is the ability to manage your own emotions when necessary and helping others to do so.

Controlling

Controlling ensures there is effective and efficient utilization of organizational resources in order to achieve the planned goals.[1] Controlling serves two basic purposes: facilitating coordination and help in planning

Controlling has 5 characteristics.

- **An end function** – comes into play when performances are made with plans
- **Pervasive** – includes all managers, all levels, all types of concerns
- **Forward looking** – look to the past for effective control in the future
- **Dynamic** – continuous review, making changes wherever possible
- **Related to planning** – without planning, controlling is meaningless and without controlling, planning is useless

What are the 4 steps in controlling?

1) Establish standards (targets to achieve)
2) Measure performance
3) Compare actual and standard performance – identify causes and extent of deviation
4) Take remedial actions – take corrective measures for deviations and possibly revising targets. A manager can only exercise control by taking corrective measures

As a manager, quality improvement steps would occur during planning and controlling.

Note that while the functions may occur in a certain order, they are not necessarily stepwise. A manager might do several functions within one day, one week, or in one project.

Write out the 5 characteristics of controlling and the 4 steps in controlling.

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Study Guide Questions

Answer these study guide questions using the study guide text. These questions will be your first pass at applying the knowledge you've read and practiced in example calculations in a question-and-answer format. Instead of answering them from memory, go to the text and your notes.

1. Are the main management functions the same from department to department? In nursing and foodservice? In accounting and medical?

2. Do the management functions help you create an approach on how objectives are achieved?

3. What management skill is the most important at higher levels of management?

4. Contrast hard skills and soft skills. If you are hiring a candidate, would you rather have someone with technical expertise but poor communication, or good soft skills who needs to learn the "tricks of the trade"?

5. What is human resources planning?

6. Delegation is a key part of which management function?

7. When should a new employee be given their performance objectives?

8. Why should there be a standardized new employee training that all employees go through?

9. In what management function or functions would quality improvement activities occur?

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Study Guide Questions (Answers)

1. Are the main management functions the same from department to department? In nursing and foodservice? In accounting and medical?
    - Yes, management functions are pervasive.

2. Do the management functions help you create an approach on how objectives are achieved?
    - No, they help set plans and objectives, but they don't outline how objectives will be achieved.

3. What management skill is the most important at higher levels of management?
    - Conceptual skills are the most important at higher levels of management.

4. Contrast hard skills and soft skills. If you are hiring a candidate, would you rather have someone with technical expertise but poor communication, or good soft skills who needs to learn the "tricks of the trade"?
    - Employers often prefer to hire candidates with exemplar soft skills because they are typically good workers, team players, and able to learn the technical skills they lack. In contrast, a candidate with exemplar technical skills but poor communication, inability to work in a team, or poor work ethic would be difficult to work with and that difficulty would outweigh the benefit of them knowing the hard skills. Obviously, the ideal candidate has both soft and hard skills.

5. What is human resources planning?
    - A key part of the planning management function, human resources planning includes forecasting staffing needs and analyzing labor market conditions to ensure that your department or organization has adequate labor requirements met.

6. Delegation is a key part of what management function?
    - Organization.

7. When should a new employee be given their performance objectives?
    - On the day they are hired.

8. Why should there be a standardized new employee training that all employees go through?
    - A standardized program means all employees are familiar with their expectations and reduces the chance of miscommunication. Standardized new employee training and reinforced personnel policies serve the same function. Both are important.

9. In what management function or functions would quality improvement activities occur?
    - Planning and controlling.

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The Healthcare System

Accreditation Organizations

Hospital accrediting agencies must acquire "deeming status" from the Centers for Medicare and Medicaid Services (CMS) per the Social Security Act. *Accreditation* is "a process by which an impartial organization reviews a company's operation to ensure the company conducts business in a manner consistent with national standards."[5] CMS does not require accreditation for healthcare delivery professionals to receive Medicare and Medicaid reimbursement.[6] Some states may have regulatory requirements that may require accreditation. Some accreditation agencies serve health insurance companies, some serve human and health organizations, and others serve healthcare delivery organizations, including hospitals. This section will focus on accreditation agencies for the last category.

The Joint Commission (TJC) was established in 1951 and is an independent, not-for-profit organization that accredits and certifies hospitals, home care organizations, rehabilitation center, long term and nursing facilities, behavioral health organizations, addiction treatment and recovery programs, ambulatory care providers, as well as clinical laboratories.[7] TJC has had deeming authority since 1966. The governing board is made up of healthcare practitioners, public health professionals, and public representatives. The TJC accreditation standards are developed according to National Patient Safety Goals and input from stakeholders and exceed the standards of the CMS Conditions of Participation (CoPs). The TJC accreditation survey process is patient-centered and focused on the healthcare delivery process on-site surveys are conducted every 3 years with annual self-assessments prepared by hospitals.

DNVGL Healthcare, Inc. is a corporation and has had deeming authority for hospitals in the US since 2008. Their accreditation requirements relate directly to the CMS CoPs with a focus on continuous improvement processes and implementing CMS quality initiatives. DNVGLHC conducts annual surveys to validate Medicare compliance. DNVGLCHC is led and managed by healthcare operational experts, including leaders in healthcare management, clinical services, health law, ISO certification, and engineering.

The National Committee for Quality Assurance (NCQA) is a non-profit established in 1990 to improve healthcare through evidence-based methods and identifying organizations that are run in ways which improve care. They accredit health plans and Patient-Centered Medical Homes and have moved to measuring the quality of medical providers using their Healthcare Effectiveness Data and Information Set (HEDIS) which includes data on 60% of the US population and provides data for accountability and benchmarking.[8]

CMS has a role in *certifying* hospitals, meaning that they meet federal regulatory standards and thus are eligible for reimbursement from federally funded programs (e.g. Medicare). If a hospital is accredited by an organization with deeming authority, it is also certified by CMS, because it has met or exceeded CMS standards. However, an organization does not need to be accredited to be CMS-certified. CMS conducts investigations and surveys (through state agency partners), including random validation surveys of accredited hospitals, to ensure compliance and eligibility. Why would an organization pay a fee to be accredited when they could be certified for free by a

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state agency on behalf of CMS? The answer: *time is money.* The organization would have to wait for an unannounced Medicare certification survey, and during that time, they are ineligible for Medicare reimbursement. Alternatively, they can pay an accreditation agency with deeming authority from CMS, and receive eligibility sooner.[9]

Value-based Healthcare

Volume-based care (or fee-for-service care) is a payment model where provider reimbursements are tied to the quantity of services and procedures. This model incentivizes health systems to provide more treatments to more patients rather than providing patients with only the care they need or by focusing on high-quality treatment.

In 2007, there was renewed interest and dedication in moving towards value-based care, with an initial focus on **Patient-Centered Medical Homes**, which center on team healthcare approaches in primary care organization and delivery.[10,11]

The Affordable Care Act (ACA) created a new division in CMS – the Center for Medicare and Medicaid Innovation (CMMI) – which was created to develop and test **Alternative Payment Model**s (**APMs**). APMs are “alternative” to traditional fee-for-service care.

One of the first APMs was **accountable care organizations (ACOs**), which are groups of healthcare professionals and/or hospitals who voluntarily come together to coordinate high-quality care to Medicare patients. Their goal is to prevent medical errors and avoid unnecessary health care duplication. Provider reimbursements are tied to quality metrics and reduced costs of care.

During the COVID-19 pandemic, medical providers using APMs were better able to manage financial volatility compared to those using traditional FFS payment models.[11]

As of 2016, the CMMI had explored 10 APMs that help advance the healthcare system from traditional fee-for-service care towards value-based care by tying 30% of traditional Medicare payments to quality or to value.[12]

Examples include the Medicare Shared Savings Program (MSSP) created by the ACA, Next Generation ACOs for ACOs experienced in population health coordination, the Comprehensive ESRD Care Model which creates ACOs that include dialysis facilities, nephrologist, and other providers and suppliers to coordinate care with accountability, Bundled Payments for Care Improvement which targets 48 conditions within a single payment for care episodes, among others.

Credentialing and Licensure

Some states in the US have licensure laws for dietitians, meaning that to practice in that state, an RD needs to be issued a license by the appropriate state agency. These statues include a **title protection provision** which prevents unlicensed individuals from presenting themselves to the public as a dietitian.

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Labor Unions and Labor Legislation

What is a labor union? A labor union "is an organization of workers joined to protect their common interests and improve their working conditions…[and serve] as an intermediary between the employer and employees."[13] Labor unions in the US are legally sanctioned and formally organized. National and international labor unions often have union "locals" at the state level or smaller geographical level with charters from the larger national or international organization. Locals may be based on geography or company employment and are typically numbered. Locals have governing bodies, including an executive board, that communicate and uphold the interests of the larger union. Unions are run by paid staff, with their salary provided for by union dues. Union dues are also used for legal costs, negotiation costs, arbitrators' fees, strike funds, and political activity.

What is the purpose of a labor union? The primary purpose of labor unions is to give workers the power to negotiate for favorable hours, wages, and working conditions through *collective bargaining.*

- **Collective bargaining** occurs when representatives from the employer (typically employees from the Human Resources Department) and representatives from the union meet to negotiate about employment conditions, wages, and hours.[13] It is called collective bargaining because the union representatives are bargaining on behalf of the union members. This situation is in contrast, for example, to a dietitian negotiating with their supervisor about a promotion for only themselves.

- Collective bargaining results in a **collective bargaining agreement** (CBA) that outlines the terms of employment for employees that are a member of that labor union/bargaining unit.[13] The CBA is enforced and monitored by the union, with complaints registered and addressed through established grievance procedures and arbitration. Management cannot change working conditions without negotiating with union representatives.[13]
    - **At-will employment** means that an employer can fire you or adjust your working conditions at nearly any time for nearly any reason.[14] At-will employment is in contrast to positions where an employer must provide **just cause** (as defined by the CBA) for termination or discipline. [13]

What laws govern labor unions?

- **The National Labor Relations Act (NLRA**, also called the Wagner Act) was passed in 1935 and guarantees rights of private sector employees to organize and join labor/trade unions, engage in collective bargaining, select their representation, and to strike if necessary. The NLRA protects an employee's right to join a union as well as their right to *not* join a union. The act also created the National Labor Relations Board which monitor the relationship between employees, unions, and employers and enforce the NLRA. This federal act permits states to pass 'right to work' laws (also called 'open shop laws'). State laws govern public sector unions. *This act is pro-labor.*
    - You should be familiar with the terms closed shop, open shop, agency shop, and union shop (next page).

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  - Under the NLRA, employers can express a *preference* that employees not unionize but employers cannot say that employees *cannot* unionize.
  - Under the NLRA, unions must provide "**duty of fair representation**" and protect employee rights as outlined in the union contract.
  - The act was extended in 1974 to include employees of private nonprofit hospitals and nursing homes[15] with amendments that unions representing workers in these industries provide 90 days' notice before terminating labor agreements and that labor disputes in these industries must be submitted to mediation by a federal or similar agency to avoid strikes that would affect essential services.[16]

- **The Taft-Hartley Act (Labor Management Relations Act)** was passed in 1947 and added a list of "unfair labor practices" on the part of unions to the NLRA. This act outlawed closed shops and permitted union shops in certain scenarios.[17] It also allows for the US president to stop national strikes for up to 80 days. This act addressed issues like restraining employee rights, discriminating against employees due to union membership, refusing to bargain in good faith, encouraging striking to force union matters, and charging excessive union fees. *This act is pro-management.*

- **Labor-Management Reporting and Disclosure Act** (also called the Landrum-Griffin Act) was passed in 1959 and, like the Taft-Hartley Act, also amended the NLRA. It was passed to regulate the internal affairs of labor unions by imposing a code of conduct in response to corruption, racketeering, and other misconduct common in unions during the 1950s.[18] For example, it protected union members from unions misusing dues and pension funds.

Write out the name of the previously discussed 3 labor union laws in the order in which they were passed. Think of ways to remember what each act was about. What was the purpose of each act? Which were considered pro-labor and which were considered pro-management?

Terms to know

- A **bargaining unit** is a group of employees with clear and identifiable common interests. A union may be made up of a single bargaining unit or a union may be made up of several bargaining units. A single labor union must represent the bargaining unit, meaning that a single bargaining unit cannot be split between different unions. Some job positions may be classified as 'bargaining unit positions' meaning that the job is represented by a labor union and that union negotiates the conditions of that job. If you apply and are chosen for that position, you would not need to join the union. Conversely, however, if you want to join a union, you must be in a recognized bargaining unit.[19]

- **Closed shops** refer to situations where only union members can be hired for positions covered by a bargaining unit. Closed shops were outlawed in 1947 by the Taft-Hartley Act.[13]

- **Union shops** refer to situations where an employee doesn't need to be a union member to be hired but must join the union within a certain time period after beginning work. [13] Union shops are illegal in some states

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- **Agency shops** refer to situations where you do not need to be a union member to hold a bargaining unit position but you have to pay partial union dues and fees. [13] The money you pay only covers union activities that you benefit from, such as collective bargaining and grievance adjustments, but not political activity or union organization.
    - **Grievance adjustment** refers to the right of employees to present grievances to their employer and have conditions adjusted as long as those adjustments are not in conflict with the collective bargaining agreement.[20]

- **Open shops** are situations where no one can be compelled to join (or not join) a union or to pay dues but still is considered part of the bargaining unit and benefits from union activities on behalf of that bargaining unit. [13] Open shops are typically in "right to work" states which is named as such because any employee has the "right to work" regardless of union membership.

- **Arbitration** is a third-party intervention where the appointed arbitrator has the power to dictate settlement terms, guaranteeing solutions. Mediation stands in contrast to arbitration where mediators have no power to dictate a final ruling.

Other labor-related legislation

- The **Fair Labor Standards Act (FLSA),** originally passed in 1938, established a Federal minimum wage, overtime pay eligibility, and child labor standards affecting both full-time and part-time workers in private and public sector positions.[21] This Act has been amended several times in order to raise the Federal minimum wage and extend/repeal protections to certain employee groups. The Act established recordkeeping regulations requiring employers to keep records of wages and hours worked. [21]
    - The Equal Pay Act of 1963 amended the FLSA to make it illegal to pay workers lower wages strictly on the basis of sex.
    - The 1967 Age Discrimination in Employment Act prohibited employee discrimination against persons 40 years of age or older.
    - The 1983 Migrant and Seasonal Agricultural Worker Protection Act provided pay, and working condition protection for migrant and seasonal farm workers.
    - The Patient Protection and Affordable Care Act amended a section of the FLSA to require employers to provide a reasonable break time for nursing mothers.[21]

- The **Civil Rights Act of 1964** ended segregation of public places and under Title VII, prohibits employment and union discrimination on the basis of race, color, religion, sex, and national origin.[22] It also created the Equal Employment Opportunity Commission which can file lawsuits on behalf of aggrieved employees.
    - The Civil Rights Act was instrumental in paving the way for the Voting Rights Act of 1965 and the Fair Housing Act of 1968.
    - Title VII Equal Employment Act has been supplemented by additional acts to extend protection from discrimination to pregnancy (1978), age (1967), and disabilities (1990)

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Table 1. Employer strategies and union strategies

	Employer Strategies	Union Strategies
Legal	Lockout: management prevents union employees from entering. Rarely used due to risk of negative publicity Sell and close: threat of job loss from the company being sold used to force compromise Strikebreakers: non-union workers brought in until striking employees come to terms, called 'scabs' Management-run operations: administrative personnel perform the jobs typically done by union members when they are striking Supervisors can communicate to employees a preference that they do not unionize[23]	Strike: employees do not go to work in order to protest working conditions, wages, etc. Often combined with picketing. Picket: employees publicly air complaints via demonstration outside employer building Boycott: refusal to do business with a party or provide services until demands are met Collective bargaining
Illegal	Blacklist: circulate list of union members in order to discriminate against them Yellow-dog contract: having employees sign an agreement that they will not join a union Hot cargo agreement: arrangement between an employer and union where the employer agrees to stop doing business with another employer[24] Refusal to participate in collective bargaining	Featherbedding: attempting to have employers pay for work that is not performed Hot cargo agreement (see lefthand cell) Collective work stoppage at health care institutions without providing required notice[24] Refusal to participate in collective bargaining

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Study Guide Questions

Answer these study guide questions using the study guide text. These questions will be your first pass at applying the knowledge you've read and practiced in example calculations in a question-and-answer format. Instead of answering them from memory, go to the text and your notes.

Q1. The following are term definitions. Write the number on a scrap piece of paper and then the correct term.

1. A situation where only union members can be hired for a particular job.
2. A group of employees with common interests that are clear and identifiable, such as performing the same job duties.
3. A situation where an employee does not need to be part of a union to hold that job, but may need to pay dues and fees to a union.
4. An intervention where the appointed third party has the power to dictate settlement terms.
5. An organization of workers joined to protect common interests and improve working conditions.
6. An intervention where the appointed third party does not have the power to dictate settlement terms.
7. A labor strategy that utilizes the unity and size of the bargaining unit to achieve better working conditions for employees.
8. A situation where a non-union employee can fill a position but must join the union within 30 days of being hired.
9. A contract that outlines terms of employment for bargaining unit members and reflects the agreement between the management representatives and union representatives.
10. A situation where all employees are considered part of the bargaining unit regardless of union membership.

Q2. The statements below correspond to which pieces of legislation? Write out the numbers below with your answer on a scrap piece of paper.

1. Also called the Wagner Act
2. Also called the Taft-Hartley Act
3. Also called the Landrum-Griffin Act
4. Ended segregation in public places
5. Created the Equal Employment Opportunity Commission
6. Made it illegal to pay workers different wages on the basis of sex
7. Prevents unions from charging excessive union fees
8. Created the National Labor Relations Board
9. Permits states to govern public sector unions
10. Provides employees with the right to join or not to join unions
11. Made "closed shops" illegal
12. Prevents unions from misusing union fees
13. Imposed a code of conduct upon labor unions
14. Outlawed child labor
15. Established a Federal minimum wage
16. Requires that unions provide duty of fair representation

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Study Guide Questions (Answers)

Q1. The following are term definitions. Write the correct term next to the definition.

1. A situation where only union members can be hired for a particular job. – Closed shop
2. A group of employees with common interests that are clear and identifiable, such as performing the same job duties. – Bargaining unit
3. A situation where an employee does not need to be part of a union to hold that job, but may need to pay dues and fees to a union. – Agency shop
4. An intervention where the appointed third party has the power to dictate settlement terms. – Arbitration
5. An organization of workers joined to protect common interests and improve working conditions. – Labor union
6. An intervention where the appointed third party does not have the power to dictate settlement terms. – Mediation
7. A labor strategy that utilizes the unity and size of the bargaining unit to achieve better working conditions for employees. – Collective bargaining
8. A situation where a non-union employee can fill a position but must join the union within 30 days of being hired. – Union shop
9. A contract that outlines terms of employment for bargaining unit members and reflects the agreement between the management representatives and union representatives. – Collective bargaining agreement
10. A situation where all employees are considered part of the bargaining unit regardless of union membership. – Open shop

Q2. The statements below correspond to which pieces of legislation?

1. Also called the Wagner Act – National Labor Relations Act of 1935
2. Also called the Taft-Hartley Act – Labor Management Relations Act of 1947
3. Also called the Landrum-Griffin Act – Labor-Management Reporting and Disclosure Act of 1959
4. Ended segregation in public places – Civil Rights Act of 1964
5. Created the Equal Employment Opportunity Commission – Civil Rights Act of 1964
6. Made it illegal to pay workers different wages on the basis of sex – Equal Pay Act of 1963, amended the Fair Labor Standards Act of 1938
7. Prevents unions from charging excessive union fees – Labor Management Relations Act of 1947
8. Created the National Labor Relations Board – National Labor Relations Act of 1935
9. Permits states to govern public sector unions – National Labor Relations Act of 1935
10. Provides employees with the right to join or not to join unions– National Labor Relations Act of 1935
11. Made “closed shops” illegal– Labor Management Relations Act of 1947
12. Prevents unions from misusing union fees – Labor-Management Reporting and Disclosure Act of 1959
13. Imposed a code of conduct upon labor unions – Labor-Management Reporting and Disclosure Act of 1959
14. Outlawed child labor – Fair Labor Standards Act of 1938
15. Established a Federal minimum wage – Fair Labor Standards Act of 1938
16. Requires that unions provide duty of fair representation– National Labor Relations Act of 1935

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Mintzberg Management Roles

Managers fill many different roles throughout the day. They switch roles as tasks, situations, and expectations require. These roles include **interpersonal, informational, and decisional.** [1]

Interpersonal roles provide information and include the roles of a figurehead, leader, and liaison.[25] Figureheads have social, ceremonial, and legal responsibilities; they serve as an authority figure and a source of inspiration. Leaders serve as a leader, and manage performance and responsibilities. Liaisons communicate with internal and external contacts and network effectively.

Informational roles process information and include the roles of a monitor, disseminator, and spokesperson.[25] Monitors seek information related to their industry, look for changes in the industry environment, and monitor the team on productivity and well-being. Disseminators communicate useful information to teams and colleagues. Spokespersons represent and speak for their organization, and transmit information about their organization and goals to people outside of it.

Decisional roles use information and include the roles of entrepreneur, disturbance handler, resource allocator, and negotiator.[25] Entrepreneurs create and control change (including problem solving), generate new ideas, and implement them. Disturbance handlers take charge when the team hits an unexpected roadblock and mediate disputes within the team. Resource allocators determine where organizational resources should be applied, allocate funding, and assign staff and other resources. Negotiators take part in and direct important negotiations within the team, department, and even organization.

From the Mintzberg's Management Roles, you can see how the management functions interplay with the roles, which in turn interplay with other management concepts we've talked about so far.

How do the management roles fit in with the 5 management functions? Some of the wording of the management roles are even similar to the language for the management functions. For example, "resource allocator" fits with both planning and controlling.

Use the Mintzberg's Management Roles worksheet to digest the text on this page.

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Leadership

What is a leader?

What is an **effective** leader? An effective leader is one who accepts that change will happen (and that change is a good thing) and works with others on their team to facilitate the *pace* and the *outcome* of that change.[1] By working with others, and keeping a positive attitude about change, the effective leader empowers themselves as a leader and empowers their employees as teammates.

An effective leader: [1]

- **Copes with change** – accepts that change should and will happen, working with others to set the pace and outcome of that change process
- **Empowers employees** – empowers themselves and employees as a team to adapt and embrace change
- **Perseveres** through difficult processes

In contrast, an effective leader does not need to "cope with complexities" – they accept complexity as something that will happen and instead, cope with change to facilitate that environment. [1] Furthermore, an effective leader doesn't seek to be understood by their team – they should be considered an **expert** and **referent** by their employees. [1]

What Mintzberg Management Role corresponds to a leader being an expert and referent?

Not all managers are effective leaders, and within lower ranks of an organization, employees may be effective leaders without being a manager.

Just as there are different leadership styles, a single leader's style may change due to the situation – some styles are more effective when the goal of leadership is compliance, and other leadership styles are more effective when there is a time limitation.

If the leadership goal is compliance, a leader should be seen as a role model who is legitimate, an expert, competent, and of a high position in the organization. [1]

Compliance may be the goal when change has to occur to meet new external guidelines – the change has to happen, and it has to happen a certain way, so having an effective leader is key for the process to go smoothly. [1]

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Leadership Styles

Just as there are management functions and management roles, there are also leadership styles. Be familiar with these terms and their definitions. Try to envision these scenarios or relate to a manager you have had to make the text more real.

Transactional leadership[26,27]

- Focus is on supervision, organization, performance
- Goal is to maintain the status quo
- Obtains employee followers through rewards and punishments, which typically only lasts for the short term
- Effective in a crisis; effective when projects have very specific parameters

Transformational leadership[26,27]

- Connects with employees' sense of identity and dedication to the mission and organization. In doing so, enhances motivation, morale, and performance
- Aligns followers with appropriate tasks in order to improve performance
- Goal is to change the future
- Acts as a positive role model
- Effective in smaller businesses due to hands-on outreach to employees, rather than in complex or large organizations

Contingency / Situational leadership[28]

- Changes leadership approach depending on the situation
- Used when tasks and environment vary, and/or when organizations are open systems requiring careful management

Compare and contrast transactional with transformational leadership styles.

Leadership Effectiveness Model

The Hersey, Blanchard, and Johnson leadership effectiveness model (commonly abbreviated as HBL for Hersey and Blanchard leadership model) focuses on situations rather than leadership styles.[29] It is also called the Situational Leadership Model.

Effective leaders adapt their leadership styles and decision-making styles with the situation. In terms of the last section on leadership styles, you could say that an effective leader utilizes contingency or situational leadership styles.

After reading this section, fill out the HBL Leadership Style worksheet (free at www.studysmartermethod.com) or included with your PTEP purchase.

The leadership effectiveness model capitalizes on this flexibility to increase a leader's effectiveness. By adapting to the current situation and leading appropriately, a leader is effective in meeting the goal. The HBL model is centered around the leader's decision making, and thus uses an individualistic decision-making approach in contrast to a group decision making approach.[30]

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The HBL model is **adaptive** and **flexible.** Leaders consider their team, the environment, and the task at-hand when making decisions. Thus, this model treats leadership as **task-dependent** and **relationship-dependent.** Task situation refers to an employee's ability and relationship situation refers to the employee's willingness or confidence in their ability. Put in simple steps, a leader using the HBL model will:

1. Identify the category of follower (employee)
2. Adapt leadership style and behavior to match follower (employee)
3. Continue to assess follower (employee) status and adapt leadership style and behavior

In this leadership style, a leader's choice depends on the task situation, or an employee's *ability,* and the relationship situation, or an employee's *willingness* or *confidence*.[29] Employees, or groups, are called "followers" in the HBL model, and there are 4 primary situations on the continuum, described by "maturity" and "readiness". Using this, there are 4 primary leadership styles: telling, selling, participating, and delegating. In the following pages, I provide tables that describe these 4 situations and 4 leadership styles (S1 to S4), and present the HBL grid to help visualize the continuum on both axes. Then, there is a worksheet for you to fill out.

HBL Leadership Styles

	Leadership Style	Brief Description	Description
S1	Telling	Instruction, direction, autocratic	Leader gives precise and firm instructions and deadlines while closely monitoring progress
S2	Selling	Persuasion, encouragement, incentive	Leader explains goals, tasks, methods and reasons, and remains available to followers to provide support
S3	Participating	Involvement, consultation, teamwork	Leader works with followers, involving the group, seeking input, and encouraging efforts
S4	Delegating	Trust, empowerment, responsibility	Leader gives responsibility to followers to set goals, plan, and execute the plan.

When answering a question about the HBL model (likely a case study), you need to consider the follower situation and the appropriate leadership style.

Maturity Levels

There are 4 maturity levels to describe employees, or followers, that correspond to the 4 leadership styles.[31] These maturity levels are commonly abbreviated as M1, M2, M3, and M4. The "follow situations" that correspond to the maturity level and leadership style are described on a spectrum of **task behavior** and **relationship behavior.** The wording of "task behavior" and "relationship behavior" may seem a little confusing. Let's describe them, and then think about how you can remember what they mean.

Task behavior: How knowledgeable is the employee? Do they have the necessary skills? How much help will they need from the leader? You can think of task behavior as the degree of *directive behavior* required by the leader. When "task behavior" or "directive behavior" is HIGH, the employee needs to be instructed on every aspect of the task they are being asked to do. When "task behavior" or "directive behavior" is LOW, the employee is knowledgeable about the task at hand and have the necessary skills to get it done

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Relationship behavior: How excited is the employee about the task? How willing are they to engage in the task? How much motivation will the leader need to provide? You can think of relationship behavior as the degree of *supportive behavior* required by the leader. When "relationship behavior" or "supportive behavior" is LOW, the employee is self-motivated and does not require a large amount of support from the leader. When "relationship behavior" or "supportive behavior" is HIGH, the employee requires a large amount of support from the leader as they may not be confident or excited about the task

M1 are the least experienced workers and are described by some texts as "unable and unwilling", corresponding to R1 in the diagram following. The employee may be inexperienced or simply lack the skills necessary for the task. The employee will require a high level of direction from the leader at every step and because the leader is guiding them in the tasks themselves, there is a low amount of supportive behavior required. M1 maturity corresponds to R1 readiness and S1 – telling.

M2 are inexperienced like M1, but are interested in learning. The leader should adapt their style to provide more supportive behavior to match the employee's excitement about increasing their skill set. The leader needs to provide both direction and support.M2 maturity corresponds to R2 readiness and S2 - selling.

M3 are employees who have most of the skills and knowledge necessary to get the job done correctly, thus requiring little guidance on the task from the leader. They benefit from supportive behavior to enhance their confidence and build the employee-leader relationship. M3 maturity corresponds to R3 readiness and S3 – participating

M4 are employees who are confident in their ability to complete at ask and can do so independently, without direction or support from the leader. M4 maturity corresponds to R4 r4e3adiness and S4 – delegating.

Remember that the degree of task behavior (directive behavior) and relationships behavior (supportive behavior) (high or low) is from the perspective of the leader, not the employee.

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The table below highlights the maturity level and readiness level of the employee and the degree of task behavior and relationship behavior required by the leader. Re-read the definitions of the S1 through S4 leadership styles and then think about how the leader adapts the amount of explaining versus encouraging (task versus relationships behavior) when working with an employee in the different M1 through M4 situations.

EMPLOYEE			LEADER		
Maturity	**Employee Readiness**	**Employee Situation Description**	**Task Behavior**	**Relationship Behavior**	**Leadership Style**
M1	Unable and unwilling	Follower lacks experience or skill and the confidence to do the task. May also lack willingness.	High	Low	S1 Telling
M2	Unable but willing	Follower lacks ability (possibly due to lack of experience) but is enthusiastic for the work. Requires high level of supportive behavior in order to learn the skills.	High	High	S2 Selling
M3	Able but unwilling	Follower is capable and experienced, but lacks the confidence or commitment. May question the goal or task.	Low	High	S3 Participating
M4	Able and willing	Follower is capable, experienced, confidence, and committed to the goals.	Low	Low	S4 Delegating

The HBL Leadership Style and the continuums of high versus low task behavior and relationship behavior required by the leader for the different follower types is often displayed graphically in 2 ways – one, with the x-axis of task behavior going from high to low, and one with that same axis going from low to high. I have included both diagrams on the next page. Review them with the information you've just read and decide which one makes more sense to you. This leadership style can be particularly tricky to learn because the way that the authors have phrased some of the terms may not seem intuitive.

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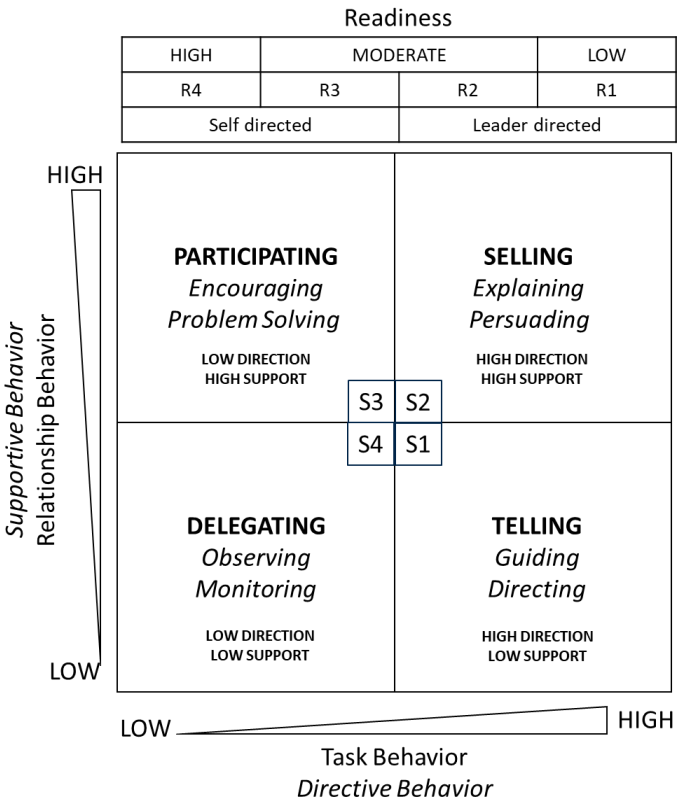
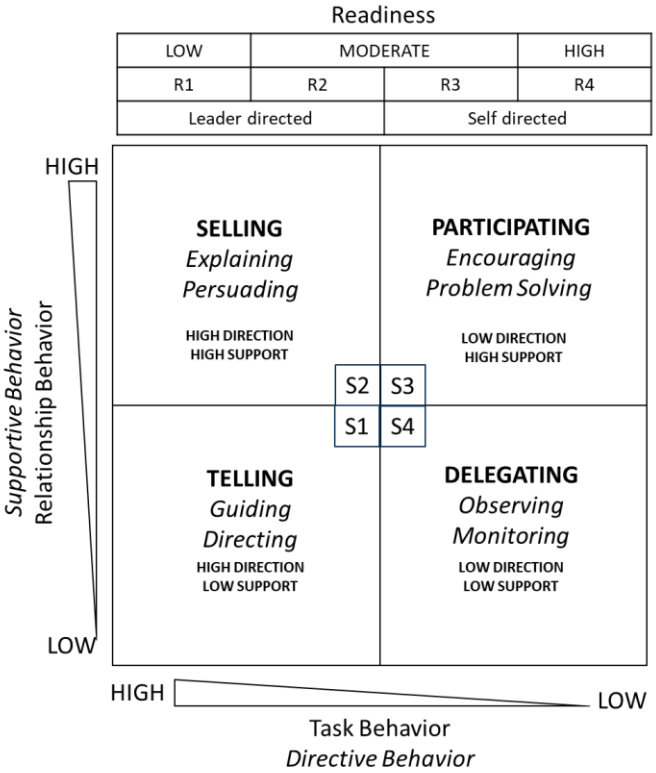


The diagram to the **left** starts with delegating in the bottom left corner, meaning that the leader can use the delegating style with employees that require low directive (task) behavior and low relationship (support) behavior from their supervisor. The leader will spend most of their time observing and monitoring the employee, because the employee is at a R4 readiness, or self-directed, readiness level. Walk through the other 3 quadrants and write out an explanation for each one like I did for delegating.

The diagram to the **right** has telling in the bottom left corner, because the axis for task behavior is going from high to low. Notice that the readiness bar is going from low to high in this version of the diagram.

I've included both types of diagrams because if you did your own research, you'd find these 2 versions. Which makes more sense to you? Use that one to study.

Remember: The level of task behavior and relationship behavior refers to what the leader must do, and corresponds directly to the matching leadership style.



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Decision Making

Decision Making Styles

There are 4 decision making styles to know for the exam: democratic, autocratic, consensus, and participative.[32]

Democratic: accepts decision made by group majority

- A foodservice manager defers to her employees to reach an agreement among themselves, and then she adopts the majority decision.

Autocratic: takes total control, makes decisions, assumes full responsibility

- Most appropriate when needing to initiate several complex, stressful, and simultaneous changes in a short period of time – when there is little time to consult with individual employees or the entire group of employees -> involves making decisions with available information

What leadership style might this go well with? Why?

Consensus: requires <u>entire</u> group to come to an agreement

Participative: leadership seeks input from others before making a management decision (note that no decision is agreed upon by the group, it's up to the manager in the end)

On a sheet of paper, list the 4 decision making styles with a brief description.

Decision Making Techniques

Game theory: introduces a competitive component into decision making by bringing the actions of an opponent into the situation. The goal is to obtain the highest gains with smallest number of losses, regardless of what a competitor does.[1]

Simulation: uses computers to imitate real-life occurrences and studies their properties, behaviors, and operating characteristics, such as that in virtual reality.[1]

Artificial intelligence: technology duplicates the thought processes of experienced decision makers[1]

Queuing theory: balancing the cost of waiting in lines against the cost of expanding facilities to prevent them. For example, balancing the number of cashiers working in the hospital cafeteria with the number of customers waiting in line[1]

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Management Concepts & Terms

The following terms are key to the organization function of management. Remember, assigning responsibility clearly so that everyone knows who they are accountable to, and what authority they have, is key to effective management. You can skip ahead a few pages to the definitions of authority, responsibility, capability, and accountability, if that would be helpful.

The 4 E's

Knowing the difference between these 4 terms is important for the exam.

- **Efficient**: getting the most output from the least amount of input[1]
- **Effective**: doing things well, producing intended results[1]
- **Economical**: having good value with respect to the money/time/effort spent[1] – aka your total inputs for the "budget"; an economical approach – aka a "thrifty" approach
- **Expedient**: suitable for a purpose, given the circumstance[1]

Stealing

Know the differences between these 5 terms that have to do with stealing.

- **Theft**: taking property without force[1] – "taking" means removing without permission or right, and is often premeditated
- **Pilfering**: to steal things of relatively little value[1] – aka "petty theft"
- **Robbery**: taking property that may involve force and person-to-person interaction[1]
- **Embezzling**: stealing money or misappropriating money belonging to the employer[1]
- **Burglary**: entering a building/residence with intention of theft. [1] Burglary does not require that property is actually stolen, and there may or may not be person-to-person interaction

Scientific Management

By optimizing and simplifying jobs, you can improve productivity that encourages employees and managers to cooperate with one another. A key aspect of **scientific management,** versus other management theories, such as continuous improvement, is that there is "**one right way**". With other types of management theories and styles, there is not only one right way.

Scientific management, also known as "Taylorism", has 4 principles[33]:

1. Replace standard procedures with a "rule of thumb" or simple habit, use the scientific method to study the work processes, and determine the most efficient way to perform specific tasks.
2. Instead of assigning workers to any job, match workers based on capability and motivation, and then train them to work at maximum efficiency.
3. Monitor work performance, and provide instructions and supervision to ensure that work is at its most efficient.
4. Allocate the work between managers and workers so managers spend their time planning and training, allowing the workers to perform their tasks efficiently.

In the 4 principles above, how is a manager optimizing jobs? Is efficiency synonymous with productivity? How are employees and managers cooperating?

Which of the 4 E terms is emphasized with scientific management?

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Upward Communication

Upward communication involves communication from the bottom of an organizational hierarchy to the top, for various reasons.[34] **Whisteblowing** uses upward communication and is key in fraud detection. Top management can also utilize upward communication, such as **exit interviews**, as a source of information in making business decisions, reengineering certain processes, and quality control. However, disadvantages of upward communication include employees fear to participate and the risk that information is changed when it goes through each tier of management, like the classic game of telephone. Think about upward communication again when we talk about organizational structure.

Management by Objective

With a management by objective outlook, the manager works with employees to review organizational objectives, formulate goals, and then track and monitor the goals that are in place. Management by objective provides control from within, established performance goals with employees, and focuses on **participative** decision making. **Remember the definition of participative** decision making? When a manager seeks input from others before making a management decision.

Delegation

Delegation allows a manager to share *responsibility* for a task.[1] Delegation is giving the appropriate *authority* to allow the employee to alter procedures in order to achieve the desired outcome. In contrast, simply telling an employee what to do or micromanaging them by requiring daily progress reports is not delegating.

Which of the HBL leadership situations would <u>not</u> be delegating?

Since delegation gives responsibility to the employee, it is important for the manager to evaluate an employee's skills (or capability) against the difficulty of the task before delegating.

What other concepts have you learned so far to evaluate an employee's skill?

The Do's and Don'ts of Delegation	
Do's	**Don'ts**
• Give precise instructions • Clarify expected results • Anticipate questions • Discuss recurring problems • Seek employee ideas about how to do the job • Be supportive and exhibit trust • Recognize good performance • Show how a process is done, and explain why it is done	• Do not threaten • Do not use a condescending attitude • Do not just give answers – illustrate the process • Do not overreact to problems • Avoid micromanagement

Delegation facilitates the management process by assigning responsibilities, delegating authority, and requiring accountability of employees via the concept of the scalar principle. The **scalar principle** is the idea that subordinates follow a chain of command and report to their immediate supervisor, not to a supervisor further up in the chain of command.[35]

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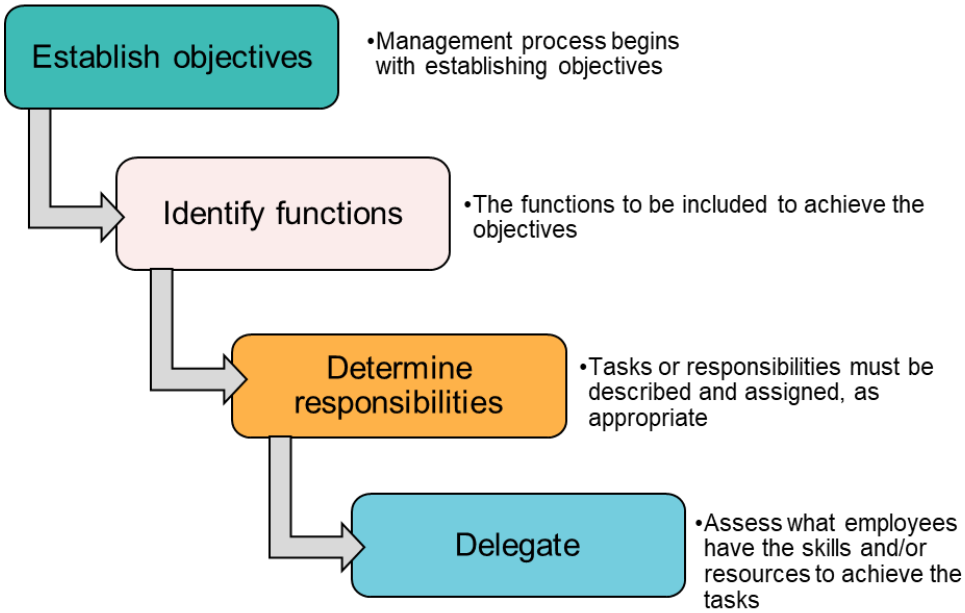
Effective delegation depends upon leadership skills, not power.

Go back to the section on decision making styles. Which decision making style would work well with effective delegation? Why?

The Management Process

The management process begins with establishing objectives and then identifies functions required to achieve the objectives. Then, the manager determines responsibilities – describes them and assigns them as appropriate. Finally, the manager delegates by assessing which employees have the skills and/or resources to achieve the tasks.



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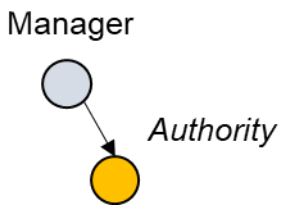
Example: Amy is the foodservice Manager at her hospital. She has received additional objectives from her director and does not have the time to accomplish them. She finds room in her budget to create a new job, hire an employee, and assign these additional objectives to the new employee. In order for the employee to successfully meet these delegated responsibilities, they need authority along with the responsibility of the tasks. The employee does not have complete control over these additional objectives - they still report back to the Foodservice Manager. Thus, the Foodservice Manager creates accountability by transferring the responsibility of these objectives to the new employee, while requiring them to meet certain standards to consider the objectives achieved. Keep this scenario in mind as we go on to define authority, capability, responsibility, and accountability.

Authority

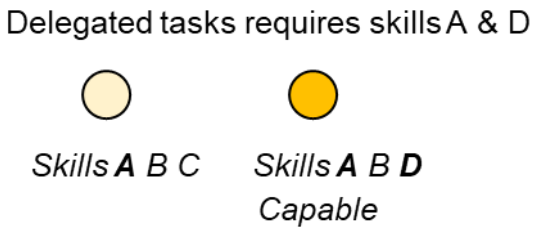
Authority is the power to make decisions, such as the decision to delegate. Decentralized and centralized organizations differ in the scope of authority at each level of management. Authority flows from the upper to lower levels of a hierarchy.



Managers have authority to delegate tasks.

Capability

Capability is the mental or physical ability to perform a task.

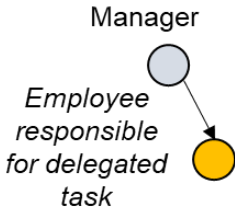


A manager can delegate tasks to employees with the required skills (and thus capability) or if time allows, train those who do not currently have the skills.

A manager should delegate tasks to employees who are capable.

Responsibility

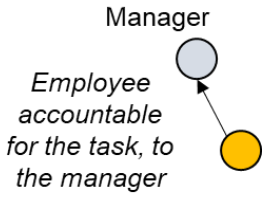
Responsibility is the obligation to perform an assigned activity, or see that someone else performs it.



Thus, both the manager and employee are responsible for this task. The tasks impart responsibility upon the employee.

Accountability

Accountability is being responsible for one's self, to an organization, or to a manager. Both the manager and employee are accountable for the task. The standards of performance impact accountability as the employee reports back to the manager.



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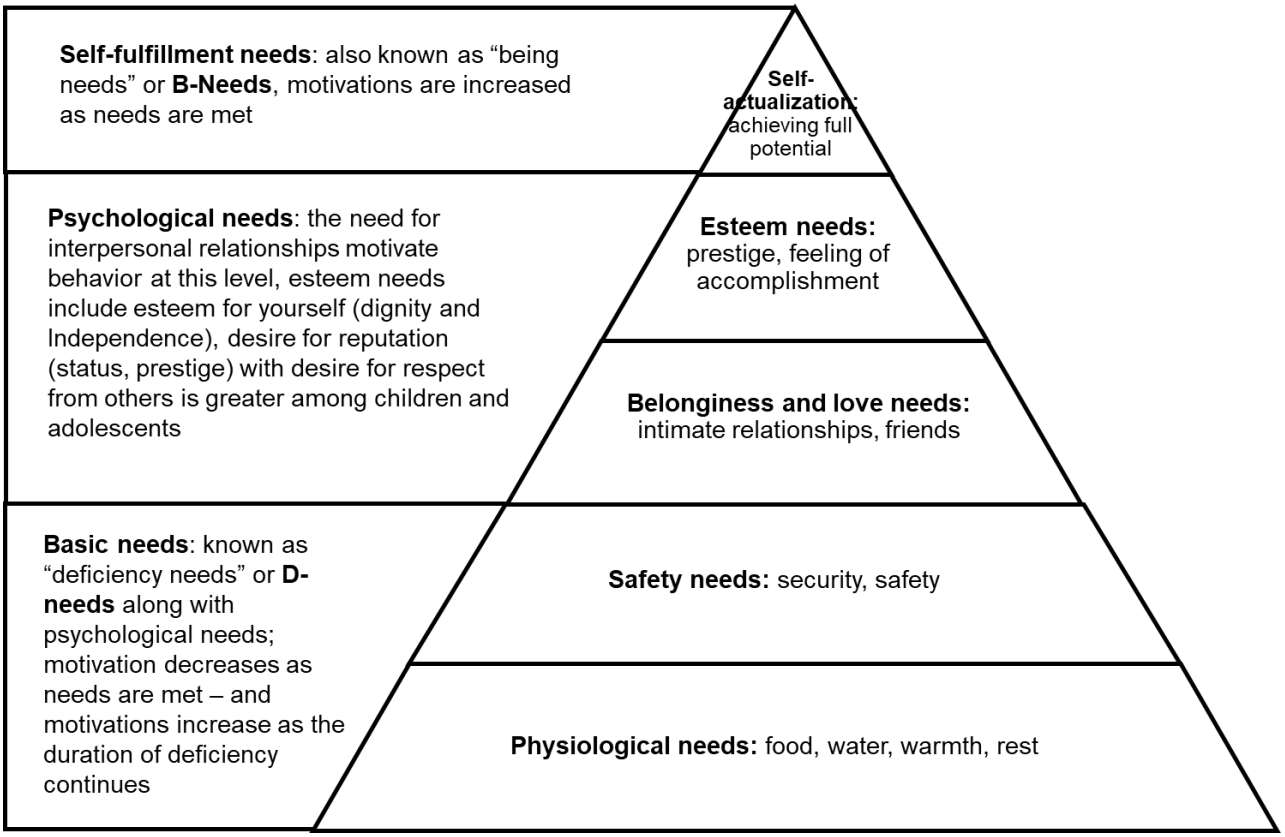
Maslow's Hierarchy of Needs

According to Maslow's theory, humans are motivated by a hierarchy of needs in that lower needs on a hierarchy must be met before achieving the needs higher up on the hierarchy. The order of needs may differ based on situations or individuals and much of human behavior is motivated by multiple needs. As you read through this page, fill out the Maslow's Hierarchy worksheet or create your own.



Basic needs include **physiological needs** (food, warmth, water, rest) and **safety needs** (security and safety). These are also known as **deficiency needs** or **D-needs** along with psychological needs. Motivations decrease as needs are met, and motivations increase as the duration of deficiency continues.

Next are **belonginess and love needs** (intimate relationships, friends), and **esteem needs** (prestige, feeling of accomplishment)– the **psychological needs,** referenced above, are the human needs for interpersonal relationships that motivate behavior at this level. Esteem includes esteem for yourself (dignity, independence), desire for reputation (status, prestige) and desire for respect from others. Esteem needs are higher among children and adolescents than certain adults.

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The highest level of Maslow's Hierarchy of Needs is **self-actualization** (achieving your full potential) also called the **self-fulfillment needs.** In contrast to **D-needs**, self-actualization includes **B-needs** (being needs). These motivations are *increased* as lower needs are met.

What are B-needs and D-needs?

Motivation increases as _____ needs are met and motivation decreases as _____ needs are met.

Deficiency needs include which 2 needs?

Psychological needs include which 2 needs?

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Management Theories

Each of these management theories draws on different aspects of Maslow's Hierarchy as underlying assumptions and motivations. Connect each listed theory with the leadership theory or style, and decision-making styles you read about in the previous section.

Theory X

McGregor's Theory X assumes that employees must be controlled, directed, and coerced.[35] Other assumptions are that employees have little ambition, they dislike work, are irresponsible, and will avoid work, if possible. Managers believing in Theory X think that the primary employee motivation is first monetary and then, security. Managers may use hard or soft management approaches. Hard management approaches include coercion, implicit threats, micromanagement, and tight controls and may result in hostility, employees producing poor work on purpose, and extreme demands from labor unions. Soft management approaches are permissive in the hopes employees will cooperate but may result in workers wanting greater reward if they can do less work.

Theory Y

McGregor's Theory Y assumes that most people not only accept but seek responsibility and that exerting physical and mental effort at work is as natural as play or rest. [35] Managers believing in Theory Y believe that most people will exercise self-control to achieve an organizational objective. Theory Y builds on the esteem and self-actualization needs from Maslow's Hierarchy of Needs. Note that managing via Theory Y does not mean a manager must use a "soft approach". Key tools for managers using Theory Y include: decentralization and delegation, broadening the scope of an employee's job description, consulting employees in decision making (participative management), and having employees set goals and conduct self-evaluations.

Theory Z of Ouchi

Theory Z of Ouchi is a "Japanese Management" style that boomed in the 1980's. Japanese employees were empowered and productive, leading to high quality products at low prices. Managerial assumptions about employees were that employees desired affiliation and expected support from their company, and that family culture and traditions are important, both at work and at home. Extensive training is a tool in Theory Z management to develop a sense of order and moral obligation to work hard and produce quality work. Managers believing in Theory Z focus on improving employee loyalty to the organization by improving their well-being, both in and outside of work. Benefits of this managerial style include stable employment (low turnover), high productivity, high morale, and high employee satisfaction.[36] Companies using this form of management typically have slow promotions, group decision-making, and life-time employment opportunities.

Divide a sheet of paper into 3 columns (or use this one) and label each column with one of the theories (Theory X, Theory Y, Theory Z). What sort of parallel descriptions did you read in this section that allow you to compare and contrast the 3 theories? Fill in your paper with descriptions, examples, and whatever else makes sense to you to understand the difference and benefits of each theory.

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Organizational Structures

Unity of Command

Unity of Command: each employee is accountable to only one superior. Considered to be a traditional or classical management approach.[35]

Scalar Principle

Scalar Principle: a management rule where each employee reports directly to their higher up and no one else. Authority and responsibility flow via delegation in a direct line, vertically from highest to lowest levels. This principle is also known as the chain of command. [35]

Line Authority

Line Authority: when multiple employees report to a single administrator, such as 20 clinical RDs reporting to one nutrition manager. There is still a chain of command but it is via line authority (multiple employees to one supervisor) versus the one-to-one in Unity of Command. [35]

Synergy

Synergy: when the outcome of multiple employees working together is greater than the sum of them working alone; you can replace the word “employees” with functional units; it’s a practical approach for getting results.[1]

Managers can foster synergy by creating teams of people who complement each other, facilitate collaboration through open work spaces, and set common objectives that let team members overcome individual differences.

Synergy can also be used in marketing and is particularly effective for small businesses or departments that lack a large budget for an aggressive marketing campaign. Synergy in marketing is when multiple low-cost initiatives are launched, rather than a single large initiative. For example, let’s say the bakery department at your hospital wants to increase sales. The manager creates a brochure with photos of their pastry trays, fruit trays, and muffin options. She sends the brochure to all hospital departments with a special offer to bundle the breakfast catering service with discounted coffee service.

Divide a sheet of paper into 4 quadrants and label each with unity of command, scalar principle, line authority and synergy. Organize the text you just read into those boxes.

Centralization versus Decentralization

In a **centralized** organization, decision making authority is centralized in upper management.[37]

Centralized organizations are good for established and detailed processes, close control of operations, uniformity of policies, practices, and procedures and a better use of centralized and/or specialized experts.

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A centralized organization has a limited amount of authority that is delegated between management levels. Decisions regarding day-to-day operations are decided upon by executives or business owners and polices are established to ensure consistency throughout the organization.

In a **decentralized** organization, decision making authority is delegated down to the lower levels of management.[37]

Decentralized organizations are good for adaptability and innovation, quick decision-making, training experience for future promotions to higher management, diverse businesses, and geographically dispersed companies.

A *decentralized* organization has a significant amount of authority delegated to lower levels of management. Upper management delegates decision-making and tasks to lower levels and maintains open communication.

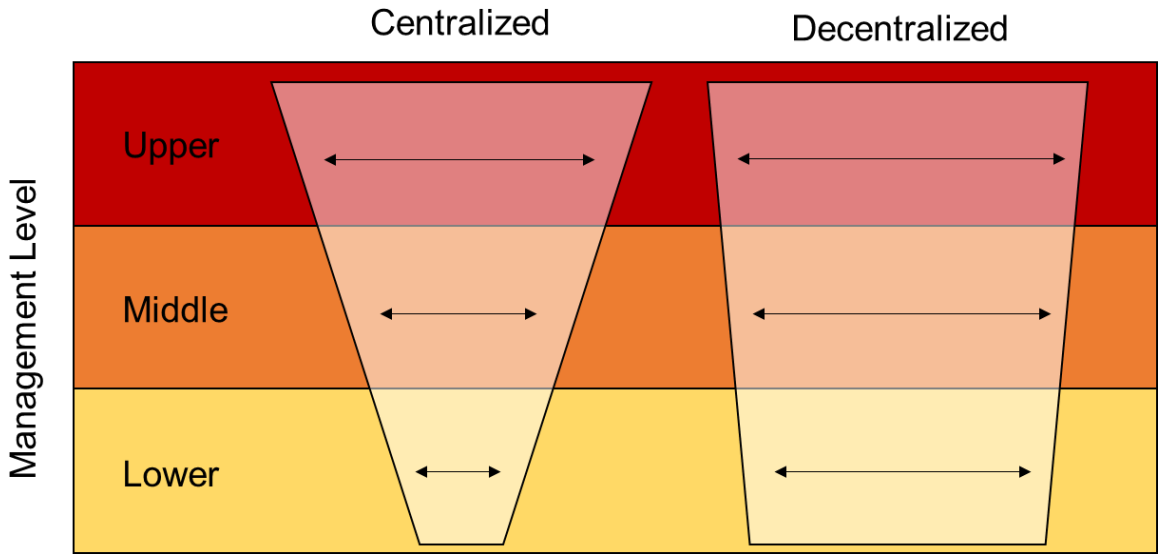
How is Theory Y related to de/centralization?

Centralization and decentralization are opposites of one another and exist on a spectrum. A company or department does not have to be fully centralized or decentralized. A start-up that needs adaptability and teamwork would do better with a decentralized structure. On the other hand, a large organization, like a pharmaceutical company with headquarters on multiple continents, would use a combination of centralized organization and decentralized organization.

Neither centralization or decentralization is considered effective or ineffective on its own – the effectiveness of the organizational structure depends on the attributes of the organization.

Scope of Authority

In centralized organizations, the scope of authority is greater at upper management levels than lower management levels. In decentralized organizations, the scope of authority is similar at all management levels. [1]



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Span of Control

Span of control is at the individual manager level, not the organizational level.[38] A manager's span of control is their ability to efficiently and effectively supervise all work sites and workers. What reduces a manager's span of control? Complicated work, geographical or departmental dispersion of workers, and vague policies.

- Define span of control.
- Would a set of standardized personnel policies increase or decrease a manager's span of control? Why?

Study Guide Questions

Answer these study guide questions using the study guide text. These questions will be your first pass at applying the knowledge you've read and practiced in example calculations in a question-and-answer format. Instead of answering them from memory, go to the text and your notes.

Use your notes and the text in this section to answer these 2 questions.

1. A friend of yours gets promoted from director of her foodservice department to regional director. She is now responsible for individual department directors across a five-state area. Will her span of control increase or decrease? Why?

2. Contrast *scope of authority* and *span of control*.

Select one of the choices between parentheses and fill in the explanation blanks when indicated.

3. A company produces infant formula and has offices across the United States. They would likely benefit from a (*decentralized or centralized)* organizational structure **because** _____**.**

4. A nonprofit health organization interacts with government officials, healthcare professionals, and education representatives. Their market span is (*diverse or not diverse)* and would likely benefit from a (*decentralized or centralized)* organizational structure.

5. A hospital's foodservice department has a Director, Nutrition Manager, Foodservice Manager, and HR Manager. The clinical RDs report to the Nutrition Manager; foodservice staff report to the Foodservice Manager; and support staff report to the HR Manager. Each week, all managers meet with the Director to get project approvals. All employees are required to follow the same policies and procedures. This department is an example of a (*decentralized or centralized)* organizational structure.

6. A large research organization has several highly qualified expert scientists and maintains several lab locations across the world, with an expert at each location. They advertise that the advantage of using their lab services is the high quality and standardized lab procedures. It would be best for them to use a (*decentralized or centralized)* *organizational* structure **because** _____**.**

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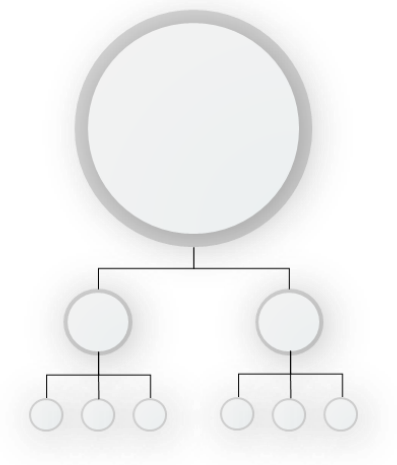
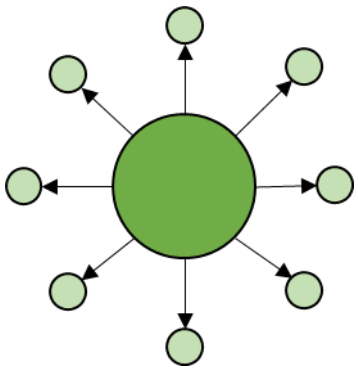
Centralized



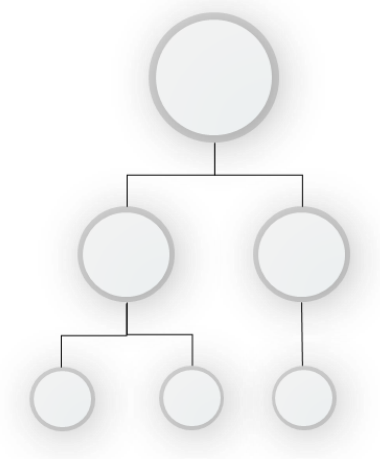
Decision making authority centralized in upper management

Good for:

- Established & detailed processes
- Close control of operations
- Uniformity of policies, practices, and procedures
- Better use of centralized and/or specialized experts



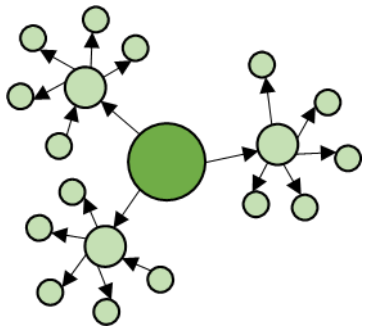
Decentralized



Authority delegated through lower levels of management

Good for:

- Adaptability and innovation
- Quick decision-making
- Good training experience for future promotions to higher level management
- Diverse businesses
- Geographically dispersed companies



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Study Guide Questions (Answers)

1. A friend of yours gets promoted from director of her foodservice department to regional director. She is now responsible for individual department directors across a five-state area. Will her span of control increase or decrease? Why?

   Geographical dispersion of workers will reduce a manager's span of control, so in this scenario, her span of control will decrease. Other factors that decrease span of control include complicated work, workers across several different departments, and vague policies that are difficult to implement or enforce.

2. Contrast *scope of authority* and *span of control*.

   Scope of authority is at the organization level while span of control is at the individual level.

3. A company produces infant formula and has offices across the United States. They would likely benefit from a (*decentralized)* organizational structure **because it is a geographically dispersed company.**

4. A nonprofit health organization interacts with government officials, healthcare professionals, and education representatives. Their market span is (*diverse)* and would likely benefit from a (*decentralized)* organizational structure.

5. A hospital's foodservice department has a Director, Nutrition Manager, Foodservice Manager, and HR Manager. The clinical RDs report to the Nutrition Manager; foodservice staff report to the Foodservice Manager; and support staff report to the HR Manager. Each week, all managers meet with the Director to get project approvals. All employees are required to follow the same policies and procedures. This department is an example of a (*centralized)* organizational structure.

6. A large research organization has several highly qualified expert scientists and maintains several lab locations across the world, with an expert at each location. They advertise that the advantage of using their lab services is the high quality and standardized lab procedures. It would be best for them to use a (*decentralized)* *organizational* structure **because of the geographically diverse locations despite having standardization and experts.**

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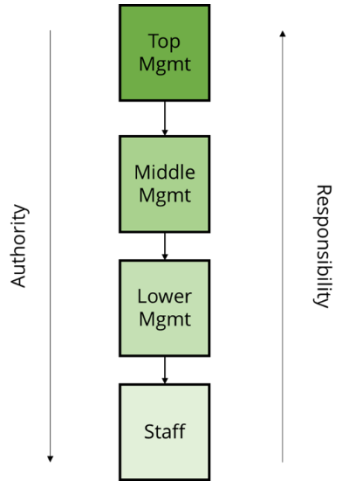
Line Organization

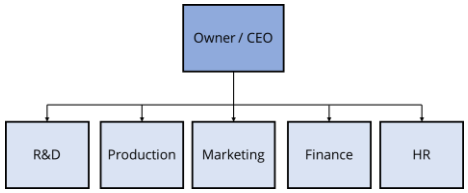


In scalar principle, unity of command, and line authority, authority flows down from top to bottom and responsibility flows up from bottom to top. All 3 of these concepts are part of line organization.

Advantages of line organization include fixed responsibility (no confusion about who should do what) and quick decision making.

Line organization has clearly defined levels of authority, responsibility, and accountability at each level of management. It is the simplest and most traditional form of organization. Even though it is the simplest form, it can be seen in the most complex organizations.

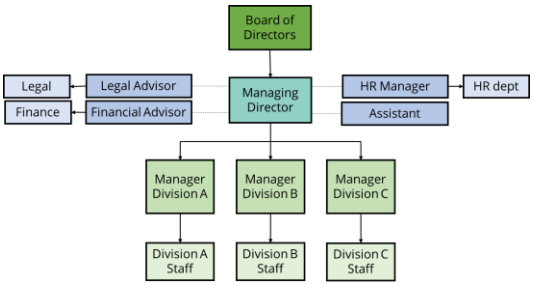
Functional Organization



In functional organization, units are divided based on specific functions like operations, finance, marketing, human resources, food service, nursing, etc.[39] Within each functional unit, there will be levels of authority. Thus, line authority can exist within other forms of organization.

Keep functional organization in mind when we talk about systems approach later on. Come back to this page and think about how healthcare systems are organized and where the foodservice department fits in.

Line and Staff Organization



Line and staff organization is a combination of line organization with shoot-offs, where staff report to various levels of management and may have more than one supervisor to report to. *Would this be line authority or scalar principle?*

Instead of functional units, line and staff organization have functional lines that are managed by a specialist supervisor. While the combination of line and staff organization is more complex than just line organization or functional organization, line and staff organization combine the *efficiency* of line organization and the *effectiveness* of functional organization.

You can imagine the diagram on the upper right being expanded to include branches for legal, finance, and HR departments with their own division managers and staff. Line authority is in the solid black arrows going down, where authority goes from top to bottom and responsibility goes from bottom to top. Staff authority is shown by the horizontal dashed lines that go from the managing director to special division managers like legal advisor, HR manager, and financial advisors.

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Practice Questions (Part 1)

Answer these practice questions using the Study Smarter Method by Veritense™ that you read about in Chapter 2 and 3 of the Study Smarter Method for the RD-Exam section. Instead of answering these questions using your memory and logic, use the study guide text and/or your own research to determine which answer is correct (and why), and why the other answers are incorrect. Pay attention to the question itself, and be sure you know what the question is asking. There are answers with explanations following. If this is your first set of practice questions, go back and skim Chapters 2 and 3 in the Study Smarter Method for the RD-Exam section to remind yourself of the types of questions and how to answer them.

1. Which of the following is not a functional subsystem of the foodservice system?
    a. Procurement
    b. Storage
    c. Sanitation
    d. Maintenance

2. Which of the following is not a management function?
    a. Staffing
    b. Delegating
    c. Planning
    d. Organization

3. Which of the following statements regarding management functions is TRUE?
    a. Management functions are used to coordinate subsystems in order to achieve objectives
    b. Staffing includes assigning duties to employees and delegating authority
    c. Controlling functions involve using different leadership styles depending on the situation
    d. Organization tasks include using problem solving to ensure resources are used properly

4. The _____ management function ensures all employees know who they are accountable to and also improves worker safety.
    a. Organization
    b. Planning
    c. Staffing
    d. Directing

5. Employee training falls under which management function?
    a. Controlling
    b. Planning
    c. Staffing
    d. Organization

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6. Which of the following scenarios is an example of progressive employee discipline?
    a. An employee has been late to work every day this week. Their manager documents the tardiness in their file, and then issues them a suspension on Friday.
    b. A new employee arrives to work with an unkempt appearance, which violates the employee dress code. Their manager speaks to them during the shift and issues a verbal warning and reviews the employee dress code with them.
    c. A supervisor sees an employee being disruptive on the patient tray line during lunch preparations. The supervisor removes them from the tray line and issues them a written warning.
    d. A dietetic intern on their foodservice rotation is conducting a quality audit of the pudding cups sold in the cafeteria. The intern notes that the weight of each pudding cup varies greatly and reports her findings to the food service manager.

7. Which of the following is the correct order of steps involved in controlling?
    a. Establish standards, measure performance, compare actual and standard performance, take remedial action
    b. Establish standards, compare actual and standard performance, measure performance, take remedial action
    c. Measure performance, establish standards, measure performance, compare actual and standard performance, take remedial action
    d. Establish standards, measure performance, take remedial action, compare actual and standard performance

8. Which of the following statements is TRUE regarding value-based healthcare?
    a. Accountable care organizations are examples of value-based healthcare
    b. Fee-for-service is another name for value-based healthcare
    c. Value-based healthcare incentivizes health care providers to increase the quantity of tests and treatments
    d. B and C

9. Which of the following statements if TRUE regarding labor unions?
    a. A bargaining unit can be spread between several labor unions
    b. Local unions can be based on geography or specific companies
    c. The National Labor Relations Act outlawed closed shops
    d. An open shop means an employee does not need to be a union member to be hired, but must become a member within a certain amount of time following their start date

10. A *leader* is an example of a _____ role while a *negotiator* is an example of a _____ role.
    a. Informational, decisional
    b. Decisional, informational
    c. Interpersonal, decisional
    d. Interpersonal, informational

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11. You are the Director of Foodservice Operations and have hired a new Associate Director. Your new AD does not have a lot of experience but shows great interested in learning the skills and knowledge necessary to excel in their new role. Which answer choice describes the HBL leadership style you should adopt with your new AD?
    a. Instructive, directing, autocratic
    b. Involved, consulting, teamwork-oriented
    c. Trusting, empowering, giving responsibility
    d. Persuasive, encouraging, incentivizing

12. You've joined a new foodservice department and learn that the department is divided into separate units for each foodservice subsystem. This organization can be described as _____.
    a. Scalar
    b. Line organization
    c. Functional
    d. Line and staff organization

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Practice Questions (Part 1) (Answers)

1. Which of the following is not a functional subsystem of the foodservice system?
    a. Procurement
    b. Storage
    c. Sanitation
    d. Maintenance

Answer: B. The 5 functional subsystems of the foodservice system are procurement, production, distribution and service, sanitation, and maintenance.

2. Which of the following is not a management function?
    a. Staffing
    b. Delegating
    c. Planning
    d. Organization

Answer: B. Delegating is part of the Organization function but is not a function itself. The 5 management functions are planning, organization, staffing, directing, and controlling

3. Which of the following statements regarding management functions is TRUE?
    a. Management functions are used to coordinate subsystems in order to achieve objectives
    b. Staffing includes assigning duties to employees and delegating authority
    c. Controlling functions involve using different leadership styles depending on the situation
    d. Organization tasks include using problem solving to ensure resources are used properly

Answer: A. *Organization* includes assigning duties to employees and delegating authority. *Directing* functions involve using different leadership styles depending on the situation. *Planning* tasks include using problem solving to ensure resources are used properly. Note that planning tasks and controlling are tightly woven together, where planning tasks are completed to ensure resources are used properly while controlling functions such as measuring performance and taking corrective action follow-up and check to see if resources are used properly.

4. The _____ management function ensures all employees know who they are accountable to and also improves worker safety.
    a. Organization
    b. Planning
    c. Staffing
    d. Directing

Answer: A. The organization management function involves identifying activities that need to be completed and delegating them, as well as assigning responsibility meaning everyone knows *who they are accountable to and what authority they have*. Successful planning and organization work together to avoid duplication of efforts (and therefore effective use of time and personnel resources). Organization is key for a safe work environment and can improve worker safety, which pays-off for the company because there are decreased costs associated with workman's compensation and lost workdays, while also building an environment that shows employee's care. *Go to pages 6 and onwards and write your own question for each of the management functions.*

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5. Employee training falls under which management function?
    a. Controlling
    b. Planning
    c. Staffing
    d. Organization

Answer: C. Employee recruitment, screening, and selection of new employees as well as training, supervising, and evaluation current employees falls under staffing.

6. Which of the following scenarios is an example of progressive employee discipline?
    a. An employee has been late to work every day this week. Their manager documents the tardiness in their file, and then issues them a suspension on Friday.
    b. A new employee arrives to work with an unkempt appearance, which violates the employee dress code. Their manager speaks to them during the shift and issues a verbal warning and reviews the employee dress code with them.
    c. A supervisor sees an employee being disruptive on the patient tray line during lunch preparations. The supervisor removes them from the tray line and issues them a written warning.
    d. A dietetic intern on their foodservice rotation is conducting a quality audit of the pudding cups sold in the cafeteria. The intern notes that the weight of each pudding cup varies greatly and reports her findings to the food service manager.

Answer: B. Progressive discipline (escalation) includes these 4 steps for formal documentation: 1) verbal warning, 2) written warning, 3) suspension, 4) termination. Answer choice A does not refer to any time that the manager has spoken with the tardy employee prior to the suspension on Friday. Answer choice C skips over the first step (verbal warning). Answer choice D does not include any information about discipline. The next step that the food service manager takes would be the scenario about discipline. Note that consistency is also key when disciplining employees – an employee should feel like they are getting the same treatment that any other employee would for the same offense. Identifying solutions to correct undesirable behaviors, providing feedback on performance, and identifying causes for unacceptable behavior are all processes involved in employee discipline specific to improving outcomes, but the most important, overarching idea is consistency.

7. Which of the following is the correct order of steps involved in controlling?
    a. Establish standards, measure performance, compare actual and standard performance, take remedial action
    b. Establish standards, compare actual and standard performance, measure performance, take remedial action
    c. Measure performance, establish standards, measure performance, compare actual and standard performance, take remedial action
    d. Establish standards, measure performance, take remedial action, compare actual and standard performance

Answer: A. The 4 steps involved in the management function of controlling is to establish standards (also known as the targets to achieve), to measure performance, then compare actual performance to the standards set in step 1, and then take remedial, or corrective, actions to correct deviations. In the last step you may also revise the standards. The true function of controlling is achieved in the least step of taking corrective action.

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8. Which of the following statements is TRUE regarding value-based healthcare?
    a. Accountable care organizations are examples of value-based healthcare
    b. Fee-for-service is another name for value-based healthcare
    c. Value-based healthcare incentivizes health care providers to increase the quantity of tests and treatments
    d. B and C

Answer: A. Volume-based healthcare is also called fee-for-service healthcare and incentivizes health care systems and providers to provide more treatments and tests for patients rather than focusing on high-quality treatment. The Affordable Care Act created the Center for Medicare and Medicaid Innovation within CMS to develop and test alternative payment models, where the 'alternative' was in contrast to the traditional fee-for-service care. Accountable care organizations, or ACOs, were one of the first alternative payment models.

9. Which of the following statements if TRUE regarding labor unions?
    a. A bargaining unit can be spread between several labor unions
    b. Local unions can be based on geography or specific companies
    c. The National Labor Relations Act outlawed closed shops
    d. An open shop means an employee does not need to be a union member to be hired, but must become a member within a certain amount of time following their start date

Answer: B. A bargaining unit is a group of employees with clear and identifiable common interests and a union might represent one *or more* bargaining units, but a bargaining unit cannot be divided between multiple unions. The Taft-Hartley Act, a pro-management act passed in 1947, outlawed closed shops and permitted union shops in certain scenarios. *What did the National Labor Relations Act of 1935 achieve?* A *union shop* refers to situations where an employee doesn't need to be a union member to be hired but must join the union within a certain time period after beginning work. Union shops are illegal in some states. *What is a closed shop, agency shop, and open shop?*

10. A *leader* is an example of a _____ role while a *negotiator* is an example of a _____ role.
    a. Informational, decisional
    b. Decisional, informational
    c. Interpersonal, decisional
    d. Interpersonal, informational

Answer: C. This question refers to Mintzberg Management Roles, which are *interpersonal* (figurehead, leader, liaison), informational (monitor, disseminator, spokesperson), and decisional (entrepreneur, disturbance handler, resource allocator, and negotiator). Managers fill different roles throughout the day and switch *roles* as the task, situation, and expectations require them to.

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11. You are the Director of Foodservice Operations and have hired a new Associate Director. Your new AD does not have a lot of experience but shows great interested in learning the skills and knowledge necessary to excel in their new role. Which answer choice describes the HBL leadership style you should adopt with your new AD?
    a. Instructive, directing, autocratic
    b. Involved, consulting, teamwork-oriented
    c. Trusting, empowering, giving responsibility
    d. Persuasive, encouraging, incentivizing

Answer: D. The scenario in the question describes an employee at M2 maturity / R2 readiness (unable but willing) which requires S2 (selling) leadership style, which involves persuasion, encouragement, and incentives where ethe leader explains goals, tasks, methods, and reasons and remains available to provide support. Answer choice A describes S1 (Telling) leadership, answer choice B describes S3 (participating) leadership, while answer choice D describes S4 (delegating) leadership.

12. You've joined a new foodservice department and learn that the department is divided into separate units for each foodservice subsystem. This organization can be described as _____.
    a. Scalar
    b. Line organization
    c. Functional
    d. Line and staff organization

Answer: C. In functional organization, units are divided based on specific functions, with line authority existing within each functional unit. Line organization utilizes scalar principle, unity of command, and linen authority with clearly defined levels of authority from top management down to staff. Line and staff organization is a combination structure that has functional *lines* of authority that exist within multiple levels of management. *Rewrite this question twice, once so the answer would be line organization and once so the answer would be line and staff organization.*

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Part 2

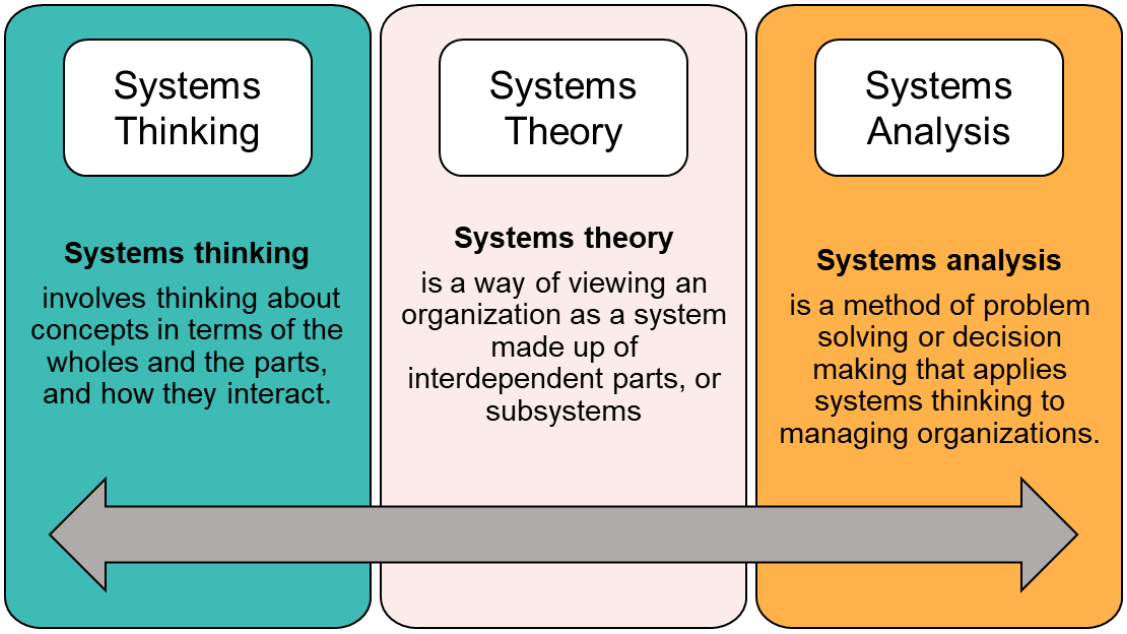
Systems Theory

A **system** is a set of interdependent parts that work together to achieve a common goal.[1]

Management is the integration and coordination of resources to achieve the desired objectives of the organization.

Organizations are open systems, influenced by and interacting with external forces in its surrounding environment. They are comprised of interdependent parts, or subsystems, that interact via feedback, operations / transformations, and in the setting of controls, management, and memory.

Don't worry about identifying how "systems thinking", "systems theory", and "systems analysis" are different. Instead, recognize that all 3 terms are key to thinking about organizations as systems. They all relate to systems thinking and theory, and you should be aware that different terms exist but refer to this overarching concept.



Six Foundational Concepts

Read about the 6 foundational concepts to systems theory on the next page to help you better understand what systems theory is. Think about how a food service manager might implement a systems approach in their management style. Don't worry about memorizing these foundational concepts. Use them to provide context.

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Six Foundational Concepts of Systems Theory

1) Organizations are a collection of parts, united by interactions and designed for accomplishing specified objectives and goals.
2) Organizations are interdependent and not self-contained, as they operate within a society.
3) Organizations are highly complex and attention should be given to the various inputs, processes, outputs, feedback loops, and general environment under which and within the organization functions.
4) Organizations constantly interact with the environment that changes them and in return, change the environment.
5) The organization cannot be understood as a function of the isolated parts, because the behavior of the system depends on how each part interacts with the others, not what each part is doing.
6) Organizations exist in dynamic equilibrium, that is, they react to internal and external changes and forces in ways that create a new state of equilibrium and balance.

According to systems theory, organizations:

- are a collection of parts united by interactions
- are designed to accomplish specific objectives
- are *interdependent* and not self-contained
- are highly complex
- constantly interact with the environment – changing both the organization and the environment, and existing in dynamic equilibrium
- must be understood by considering all parts

Takeaways

- Concepts 1 and 2: Organizations are systems, made up of subsystems, and need to be managed. *Write the definition of management from Part I.*
- Concepts 3 through 6: The interactions of subsystems, operations, and transformations, include feedback with internal and external environments.

5 Essential Processes in Systems Theory[1]

1. **DESIRED OUTPUTS:** Identify an organization's fundamental values and the goals and objectives that arise from those values
    - *Outputs* include finished products and services
2. **ENVIRONMENT:** Assessment of the organization's environment as opportunities or threats
3. **INPUTS:** Assessment of the organization's resources and capabilities
    - *Inputs* include resources like money, materials, time, personnel, facilities, and information
4. **OPERATIONS:** Identification of an organization's structure
    - Operations refer to the work done to transform inputs into outputs, also known as *transformation*
5. **MANAGEMENT:** Development of the organization's management structure
    - Functions of management (list them from Part I):
    - *Linking processes*: communication, decision making

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You should know examples of inputs, outputs, and linking processes, as well as the definitions of the key terms on the next page.

Other Terms to Know

Transformation: the processes required to change inputs into outputs

Interdependency: key to an efficient and effective system. Allows for continual relationships and interaction between the subunits within an organization.

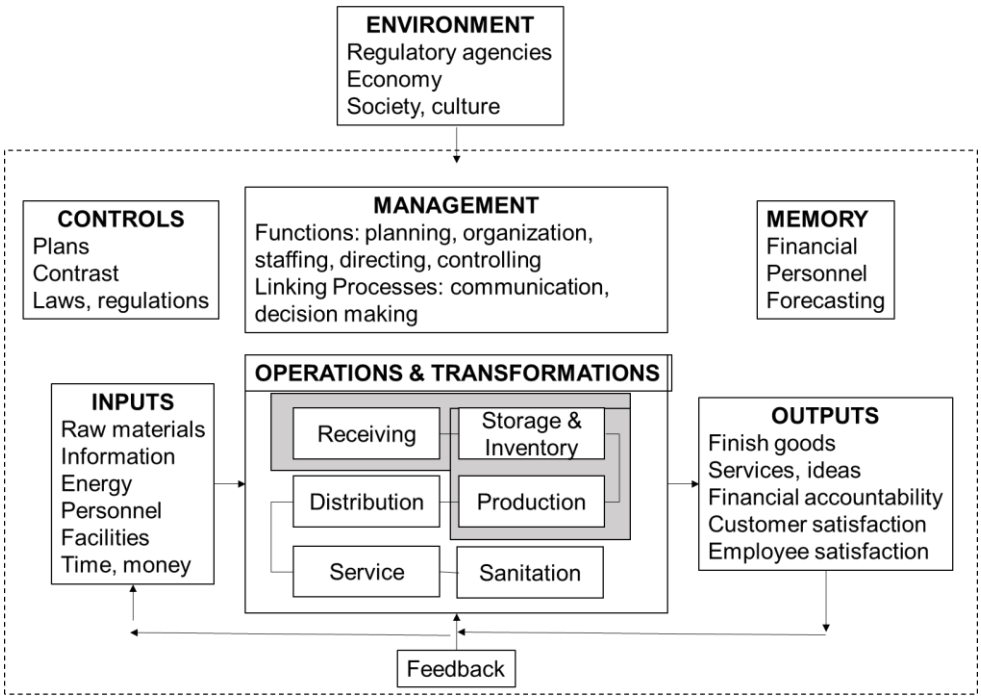
Feedback: information on how operations worked or failed, or how they should be changed or modified to restore equilibrium (can be internal or external)

Equifinality: outputs may be achieved in a variety of ways, such as producing quality meals (outputs) from both raw ingredients in-house and/or quality pre-prepared products (inputs)

Linking processes: used by managers to unify a system. Include communication and decision making.

Remember in the Part I overview, I told you we'd talk about systems approach? Note how the management part of the systems approach above mentions functions but also linking processes. Those may sound similar to things we discussed in Part I, and they are, but the theories are distinct.

Some of the terms defined in this subsection were used in the 6 foundational concepts. Take out a sheet of paper and summarize the 6 foundational concepts, highlighting the key terms and defining them. Does knowing the specific definition of those key terms, like interdependency, help you understand the foundational concepts better?



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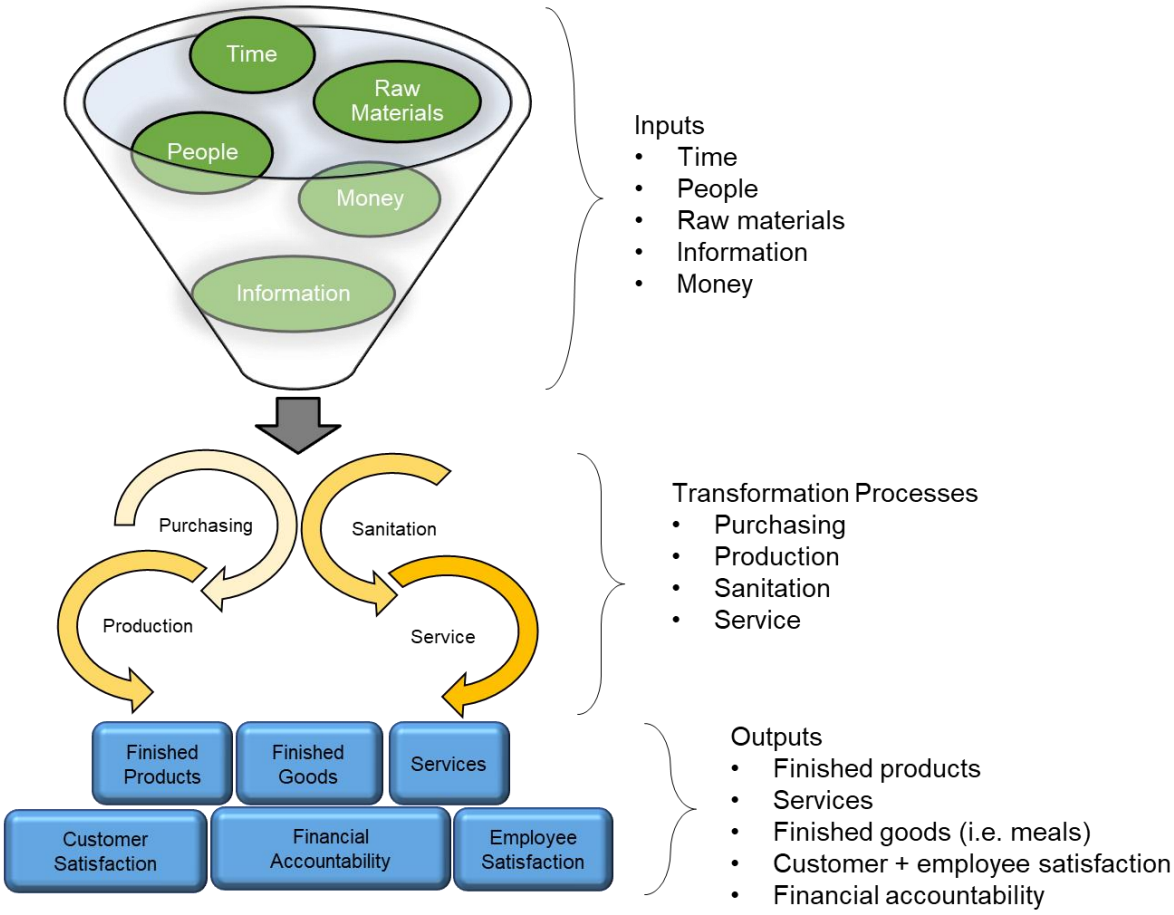
Foodservice Systems Model

The Foodservice Systems Model categorizes system components as inputs, outputs, or the transformation processes that change inputs to outputs. [1] It is an example of using a systems approach to food service management.



Subsystems and Feedback

Systems thinking is a management approach that thinks about the organizational unit as a whole when making decisions or allocating resources.

Subsystems are complete systems unto themselves but are also part of a larger system.

Departments are examples of subsystems. There can be tension and conflict when the activities of two subsystems overlap.[1] This can occur when there are no boundaries or there is a permeability of boundaries.

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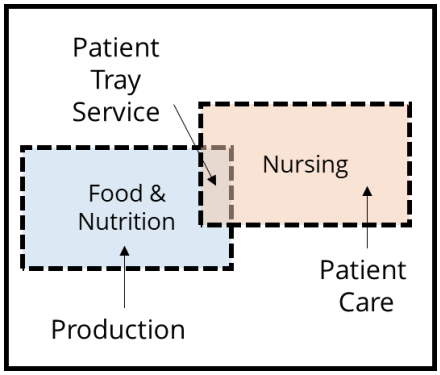
Example: Patient Tray Service

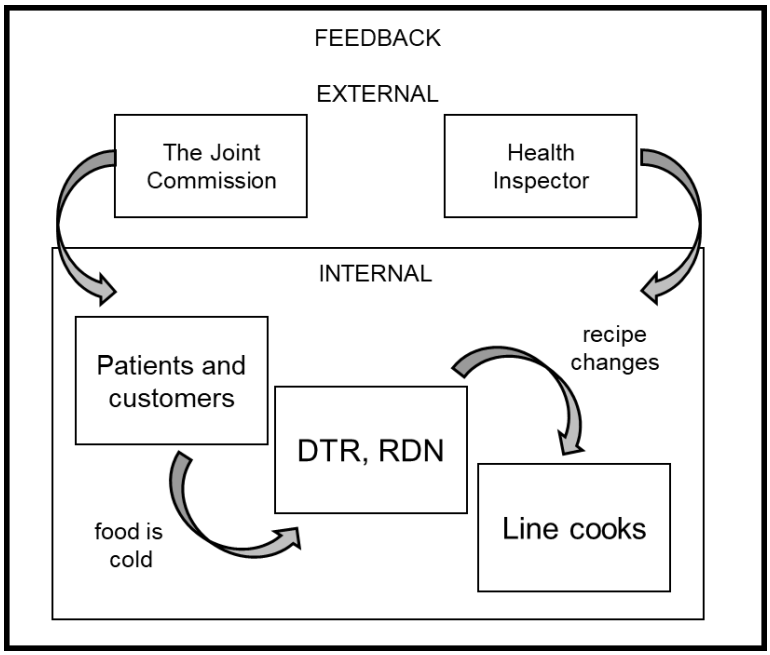
Food and nutrition is responsible for delivering patient trays and nursing is responsible for patient care. However, the interface between nursing and food and nutrition service, regarding all the details of patient tray service, isn't very clear. There are no official boundaries communicated by either department in terms of activity limitations. This situation leads to clashes between staff in the 2 subsystems.



Example: Cold Food and Feedback -The dietetic technicians and dietitians hear from the patients and cafeteria customers that the food is cold. The DTRs and RDNs relay this information to the food service manager, who discusses recipe changes with the chef, trickling down to the line cooks. Using a systems approach, evaluating the effectiveness of a new policy should consider both the internal and the external environments via feedback (See next page for a diagram).

In this example, **internal feedback** comes from the patients and customers about food temperature to the diet tech or the dietitian. Internal feedback occurs within a department, so the communication between the DTR/RDNs, foodservice manager, chef, and line cooks is also internal feedback.

External feedback, such as that from The Joint Commission or the health inspector, is external feedback. In this example, there is no external feedback. Don't be fooled by thinking the feedback from the patients and customers is external feedback.



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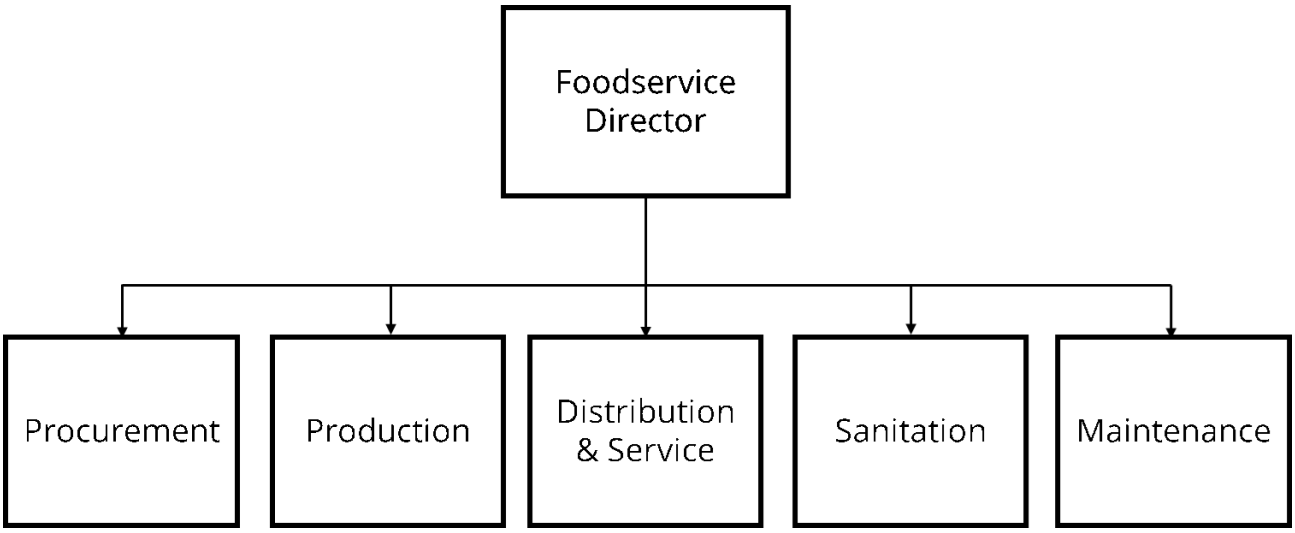
Functional Subsystems

Organizations may be divided based on function. In food service, functional units may include procurement, production, distribution and service, sanitation, and maintenance.

Transformation processes depend on the resources available, such as equipment, staff, inventory, and time. When asked about the most important resource for various functional subsystems, think about what that subsystem's purpose is and if it could function without the listed resource.[1]

Example: For a distribution and service subsystem, equipment like steam tables are key. For a catering service, chafers and similar items would be key equipment. For patient service, having staff to re-heat and serve the meals is key. Don't always think that staff is necessary – with the steam table example, technically customers can serve themselves, but without the steam table, staff wouldn't have warm food to serve.

What resources are important for the foodservice subsystems listed below?



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Study Guide Questions

Answer these study guide questions using the study guide text. These questions will be your first pass at applying the knowledge you've read and practiced in example calculations in a question-and-answer format. Instead of answering them from memory, go to the text and your notes.

1. Which of the following represent transformation processes? Circle all that apply.
    a. Following standards
    b. Establishing goals and objectives
    c. Communication
    d. Management functions
    e. Functional subsystems
    f. Linking processes

2. Judah is the manager of a hospital's foodservice department. Customer feedback over the past quarter was not very positive. For the new quarter, Judah is implementing a brand-new menu that will require more foods to be prepared from scratch. Which of the following are examples of systems thinking? Why? Answers at the end of the section.
    a. Judah needs to reorganize kitchen staff duties and schedules to account for new food production needs from the prep cooks, line workers, and dish washers.
    b. In an effort to increase department revenue and cover the cost of new equipment, Judah creates a catering menu. He distributes it to department heads, offering catering services for their monthly meetings.
    c. To accommodate the new food recipes, Judah selects a produce vendor who can deliver fresh, high-quality produce twice a day.
    d. A staff member brings up that the current cleaning chemicals used in the dishwasher are not environmentally friendly. She submits a proposal to Judah to change all cleaning chemicals over to ones that do no harm the environment.

3. Match the terms and scenarios in the left-hand column to the term in the right-hand column. Some terms may be used more than once.

• Customer satisfaction • Distribution • The health inspector provides an assessment of the dish room. • Employee satisfaction • The Food Production Manager agrees to reorganize the hot and cold food stations to facilitate productivity. • Planning and directing • The line cooks receive information from the RDN about recipe modifications. • Requirements per the Americans with Disabilities Act	• Output • Linking process • Transformation • External feedback • Management • Internal feedback • Environment

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Study Guide Questions (Answers)

1. Which of the following represent transformation processes? Circle all that apply.
    a. Following standards - no
    b. Establishing goals and objectives - no
    c. Communication - no
    d. Management functions – yes, and includes planning, organizing, staffing, directing, and controlling
    e. Functional subsystems – yes, and includes procurement, production, sanitation, distribution and service, and maintenance
    f. Linking processes – yes, and includes decision making, communication, and balance

2. Judah is the manager of a hospital's foodservice department. Customer feedback over the past quarter was not very positive. For the new quarter, Judah is implementing a brand-new menu that will require more foods to be prepared from scratch. Which of the following are examples of systems thinking?
    a. Judah needs to reorganize kitchen staff duties and schedules to account for new food production needs from the prep cooks, line workers, and dish washers.

Yes, this is systems thinking because it changes the kitchen, the staff, and allocated resources completely, and across the entire department. New recipes require new ingredients, new production schedules, and thus, new dishwashing and support staff schedules.

    b. In an effort to increase department revenue and cover the cost of new equipment, Judah creates a catering menu. He distributes it to department heads, offering catering services for their monthly meetings.

No, this is not an example of systems thinking but is a good management approach to offset the cost of new equipment needed for the new menu.

    c. To accommodate the new food recipes, Judah selects a produce vendor who can deliver fresh, high-quality produce twice a day.

No, this is not an example of systems thinking because it only addresses one component of the foodservice subsystem – ingredients.

    d. A staff member brings up that the current cleaning chemicals used in the dishwasher are not environmentally friendly. She submits a proposal to Judah to change all cleaning chemicals over to ones that do no harm the environment.

No, this is not an example of systems thinking. It only addresses one component of the foodservice subsystem – sanitation, and does not relate to the project at hand.

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3. Match the terms and scenarios in the left-hand column to the term in the right-hand column. Some terms may be used more than once.

- **Outputs** are finished products and services, and include employee and customer satisfaction.
- **Distribution** is one of several operations or transformational processes that change inputs to outputs.
- **Feedback** can be internal (from within the organization or subsystem), such as the feedback between the line cook and RDN, or external (from outside the organization or subsystem), such as the health inspector assessment.
- **Management** includes **linking processes** and the 5 functions of management, which include planning and directing.
- The Food Production Manager scenario is an example of **linking process,** not feedback, because it is an example of **communication,** which is a linking process.
- Laws and regulations from external organizations, such as those from the Americans with Disabilities Act, are considered "**environment**".

Customer satisfaction	Output
Distribution	Transformation
The health inspector provides an assessment of the dish room	External feedback
Employee satisfaction	Output
The Food Production Manager agrees to reorganize the hot and cold food stations to facilitate productivity	Linking process
Planning and directing	Management
The line cooks receive information form the RDN about recipe modifications	Internal feedback
Requirements per the Americans with Disabilities Act	Environment

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Strategic Planning

Strategic planning is, "the continuous and systematic management process to determine long-term goals, the best approach to accomplish them, and how success will be measured."[1] It is used to inform policies and operational decisions. It is not used in evaluation.

Strategic planning may also be called strategic management. It is a form of organizational and environmental analysis used to establish goals and success criteria by implementing strategies to move towards those goals. [1] A key part of strategic planning is evaluating progress towards the goal.

With strategic planning, we ask the questions:

- Where are we now?
- Where are we going?
- How do we get there?
- Are we on track?

The following are descriptions of strategic planning. Think of antonyms for these to solidify the concept of strategic planning in your mind. Contrast what it is NOT with what it IS.

Strategic planning is…	Strategic planning is NOT…
Continuous	
Systematic	
Future-thinking	
Creates a plan to accomplish outcomes	
Analyzes the organization in context of environment	
Determines success criteria	
Evaluates progress in achieving outcomes and success	

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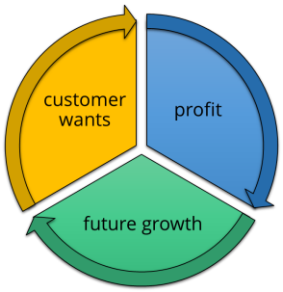
Marketing



Marketing is concerned with: [1]

- **Customer wants** – focus on producing the goods customers want
- **Profit** – maximize net profit rather than focusing on total sales
- **Future growth** – forecasting future growth

The 4 P's of Marketing

Marketing mix is an effective way to understand what products or services can offer and how to present the product successfully. There are 4 P's of Marketing (mix):[40]

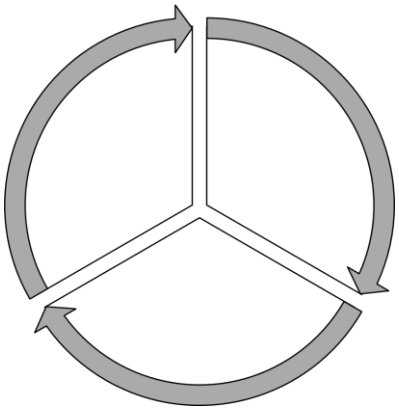
1. **Price:** pricing affects how a product is sold; linked to perceived value of product; must understand how a consumer sees the product because a price too high OR too low will hinder sales.

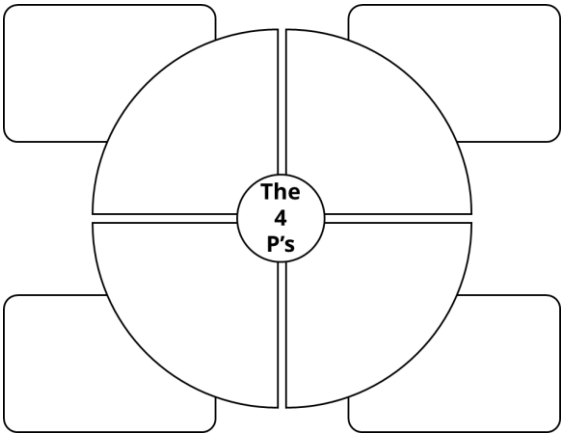
2. **Promotion**: marketing communication strategies and techniques, including advertising, sales promotions, special offers, and public relations. Must be suitable for product, price, and end user. Promotion is the communication part of the entire marketing function.

3. **Place**: how is the product provided to the customer? Placement strategy assesses the most suitable channel and must complement the rest of the product strategy.

4. **Product:** a tangible good or intangible service; developed to meet a specific consumer need and demand. Need to understand the problem the product is trying to solve and then convey the benefits to the consumer.

Fill in the blank diagrams on the Marketing Worksheet as an active learning review for the 3 points of marketing and the 4 P's of marketing mix.





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Marketing Eras

To understand marketing, it can be helpful to understand the different eras marketing has gone through.

- **Production era:** manufacturers focused on mass production
- **Sales era:** marketing strategies shifted to pricing techniques to revitalize sales as markets became crowded with product
- **Marketing era:** shift to marketing techniques
- **Technological era:** utilizes emerging technologies to reach potential customers

Market Segmentation

Marketing strategies often utilize market segmentation to design marketing campaigns. There are 4 broad ways to segment the market.

- **Geographic:** physical location
- **Psychographic:** personality, interests, beliefs
- **Behavioristic:** consumer behaviors towards products
- **Demographic:** age, sex, race, ethnicity, income, education, and occupation – they describe a person

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Marketing Lifestyle Groups

There are 8 VALS (Values and Lifestyles) Segments of American Adult consumers (Innovators, Survivors, Makers, Experiencers, Strivers, Achievers, Believers, and Thinkers). They describe mindsets by psychographic traits and demographics in how they drive consumer behavior.[41] The 2 important aspects of each term are primary motivation and resources. Their combination determines how each person expresses themselves in the marketplace as a consumer. Be able to match each term with their descriptors.

We describe VALS groups in terms of primary motivation and resources.

Primary motivations include ideals, achievement, and self-expression. Innovators are motivated by all 3 to different degrees, while survivors have no primary motivations. Thinkers and Believers are motivated by ideals. Achievers and Strivers are motivated by achievement. Experiencers and Makers are motivated by self-expression.

Resources go beyond income and include energy, self-confidence, intellectualism, novelty seeking, innovativeness, impulsiveness, leadership, and vanity. Different levels of resources improve or constrain how a person expresses their primary motivation.

Primary Motivation		
Ideals	**Achievement**	**Self-Expression**
Guided by knowledge and principles	Demonstrate success to those around them	Social activity, physical activity, variety, and risk
• Traditional codes: family, religion, community, country • Intellectual codes: order, knowledge, responsibility • Contemporary codes	• Outward expressions of achievement and success • Income and possessions	• Building, fixing, creating • Exciting experiences

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Think about how Maslow's Hierarchy of Needs fits in with these marketing lifestyle groups.

- **Innovators:** have high income, high self-esteem. Successful and sophisticated, take-charge. Abundant resources and exhibit all 3 primary motivations: ideals, achievements, self-expression
- **Survivors:** live narrowly-focused lives; believe the world is changing too quickly as they have few resources with which to cope; comfortable with the familiar and concerned with safety and security; do not have strong primary motivation because they are focused on meeting needs, not fulfilling desires
- **Makers:** have lower income; focus on self-sufficiency and family; express themselves and experience the world by working on it – building a house, raising children, fixing a car, home-canning vegetables; have the skill and energy to carry out their projects
- **Experiencers:** are young, impulsive; seek variety, excitement, and risky activities; have self-expression, enthusiasm– quickly become enthusiastic about new possibilities but equally quickly move on
- **Strivers:** trendy, fun loving, motivated by achievement and concerned about the opinions and approval of others; they define their success by money
- **Achievers:** successful individuals, typically high income, motivated by a desire for achievement, have goal-oriented lifestyles, and a deep commitment to career and family
- **Believers**: motivated by ideals, conservative and conventional, concrete beliefs based on traditional codes – family, religion, community, the nation
- **Thinkers**: motivated by ideals, mature, satisfied, comfortable, and reflective. Value order, knowledge, and responsibility.

	Primary Motivation		
	Ideals	**Achievement**	**Self-Expression**
High Resources **$$$**	Innovators		
Moderate Resources **$$**	Thinkers Believers	Achievers Strivers	Experiencers Makers
Low Resources **$**	**No primary motivation**		
	Survivors		

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Cultural Competence & Humility

On the following pages, you'll see written descriptions of several cultural competence models along with more in-depth diagrams. In the Management Worksheets you'll find blank versions of the diagrams that follow.

Cultural competence: a set of congruent attitudes, behaviors, and policies that come together; organizations and institutions implement systems to apply cultural competence into practice.[42] Shortcomings of cultural competence include the tendency to center "whiteness" as representing the norm with cultural competence a lack of knowledge of "other",[43] the implication that there is an endpoint of being "culturally competent", and the suggestion that you can apply categorical knowledge about a group to a person, which can lead to stereotyping and bias.[44]

Cultural humility: the ability to maintain an interpersonal stance that is "other-oriented" (open to the other) in relation to aspects of cultural identity most important to the client. Cultural humility requires awareness and sensitivity to historic realities (including legacies of violence and oppression against certain groups) in addition to exploring and awareness of your own personal beliefs.[43] Please read this article on cultural humility in healthcare: https://bit.ly/3wjGOnx

Cultural sensitivity is awareness of similarities and differences between different cultures.

The dominant force in how a patient views healthcare is **individual experience**.[42] Social hierarchy, race, and ethnicity are secondary drivers. There are several models of cultural competence that touch on different parts of the healthcare system.

The ETHNIC Model of culturally competent care

How does a patient's understanding of their illness and treatment fit with what is expected and accepted in their culture? How can we determine treatment options in a collaborative (not imposed) process? This model focuses on questions and conversations providers should have with patients to achieve their goals. The model stands for: **E**xplanation, **T**reatment, **H**ealers, **N**egotiation, **I**ntervention, **C**ollaboration. *(Steven J Levin, MD; Robert C Like, MD; Jan E Gottleib, MPH 1997)*

The LEARN Model

The LEARN model was designed to overcome communication barriers in health care encounters. The model stands for: **L**isten, **E**xplain, **A**cknowledge, **R**ecommend treatment, **N**egotiate agreement. (*Elois Ann Berlin, William C. Fowlkes, Jr in 1983)*

The BATHE Model

This model helps create a culturally competent environment, focusing on the psychosocial context for the patient's visit rather than medical details. The model stands for: **B**ackground, **A**ffect, **T**rouble, **H**andling, **E**mpathy. (*Marian Stuart, PhD and Joseph A Lieberman III, MD MPH*

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The ETHNIC Model

E

Explanation
- Why do you think you have these symptoms?
- If you know other people with this condition, how are they addressing them?
- What do your friends and family say about these symptoms?
- Have you heard about these symptoms in the media, such as the television or newspaper?

T

Treatment
- What kinds of medications or other remedies have you experimented with to treat your illness?
- What sorts of foods and drinks do you consume or avoid to stay healthy, or specifically, to treat this illness?
- What kind of treatment are you seeking from the practitioner?

H

Healers
- Have you sought advice about your illness from nontraditional sources?
- What did you like about this method of treatment?

N

Negotiation
- What changes do you expect to achieve with this treatment?

I

Intervention
- How do you feel about the treatment plan?
- Which components of the treatment plan are you most concerned about

C

Collaboration
- Here are the ways that other health care team members will help with the treatment plan.
- Here are some community resources and some ways that your friends and family might help you with the following treatment plan.

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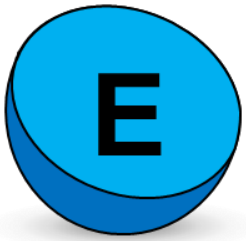
The LEARN Model

Listen – with empathy and understanding to how the patient perceives the problem. Ask open-ended questions about what they think is the illness' cause, process, duration, and outcome, and the resources that they have.



Explain – your perceptions of the problem. Explain the strategy for addressing medical complaints, particularly for patients unfamiliar with Western health care, can help avoid misunderstandings.

Acknowledge – and discuss the differences and similarities. Conceptual reconciliation of the patient explanation and the clinician explanation especially regarding causes, symptoms, and treatment for the illness.

Recommend – treatment. They should be rooted in the resolution achieved in the Acknowledge step of this process.

Negotiate – agreement. With the patient and practitioner partnered, they should develop a treatment plan agreed upon by both parties.

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The BATHE Model

B

Background – educe the reason for the patient visit. Ask "What is going in your life?"

A

Affect – provide the patient an opportunity to report how they feel about the problem. Ask "How have you been feeling about what is going on?"

T

Trouble – provide focus to the visit while still extracting the *significance* of the problem (in the patients perception). Ask "What about the situation concerns you the most?"

H

Handling – facilitates discovery of what the patient has been doing so far, which can inform the intervention course. Ask "Before now, what have you been doing to address the problem?"

E

Empathy – provide support to the patient by legitimizing their response to the problem. "This must be difficult for you".

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The GREET Model

This model is useful when working with immigrant populations and helps providers understand the context of the patient and how they understand information related to their illness. **G**eneration, **R**eason, **E**xtended family, **E**thnic behavior, **T**ime living in the United States. *(Nilda Chong, MD PhD MPH 2002)*

Acculturation *is the assimilation to a different culture.*

Acclimation *is the process or result of becoming accustomed to a new climate or conditions.*

Campinha-Bacote Model

Starting in the individual, this model states that cultural competence expands to the family and community level. The 5-step process focuses on 1) awareness, 2) knowledge, 3) skill, 4) encounter, and 5) desire.

Awareness involves challenging assumptions and asking client questions about their values, beliefs, and practices, resulting in acquisition of cultural knowledge. With **cultural knowledge**, the provider familiarizes themselves with variations in cultural impact and expands their **cultural skills** to collect key information in order to conduct effective assessments and decide on an intervention that takes into account preferred language, foods consumed or avoided (especially when feeling ill), and foods consumed or avoided for religious reasons.

This **cultural encounter** is the best time to discuss nutrition. The final step is **cultural desire**, where the practitioners want to engage in cross-cultural care. *(Joseph Campinha-Bacote, PhD 1998)*

Know the definitions of the 5 steps.

The Purnell Model

This model is derived from multiple theories and includes 16 assumptions about culture. “A culturally competent health care provider develops an awareness of his or her existence, sensations, thoughts, and environment without letting these factors have an undue effect on those for whom care is provided.” – *Larry Purnell, PhD RN FAAN.*

Explanatory Models Approach

This model “eschews the trait list approach that understands culture as a set of already known factors and instead, emphasizes engagement with others and with the practices that people undertake in their local worlds and the ambivalence that many people feel as a result of being between worlds, including those who self-identify with multiple cultural groups.” It is a 6-step “mini-ethnography” approach that has health care providers use their expert knowledge next to the patient’s own explanation and viewpoint.

(Arthur Kleinman, MD and Peter Benson, MD)

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Explanatory Models Approach

Step 1 Ethnic Identity

- Inquire as to a patient's ethnic identity
- Is it an important construct in their self-identity?
- Helps avoid assumptions that lead to stereotyping

Step 2 What is at stake?

- Evaluate concerns about relationships, resources, commitments, and life
- To better understand patients and their families in the context of their "moral lives"

Step 3 The Illness Narrative

- Ask a sequence of questions to discover how the patient defines their illness both pragmatically and personally
- "What do you call this problem?" "What do you believe cause this problem?" "What do you believe is happening inside your body as a result of this problem?"

Step 4 Psychosocial stresses

- Recognizing the patient's anxiety, financial stresses, and troubles related to work and family regarding the illness and treatment, the provider can make better recommendations to alleviate those burdens
- Illness is frequently accompanied by depression and anxiety which can worsen health outcomes

Step 5 Influence of culture on clinical relationships

- Providers should self-reflect on the "formative effect of biomedicine and institutions on the most routine clinical practices including bias, inappropriate and excessive use of technology, and stereotyping"

Step 6 Problems of cultural competency approach

- Acknowledging that one size does not fit all
- Culturally competent interventions is not always the best intervention because "attention to cultural differences can be interpreted as intrusive' or "lending excessive significance to differences misguides finding the root cause of the problem"

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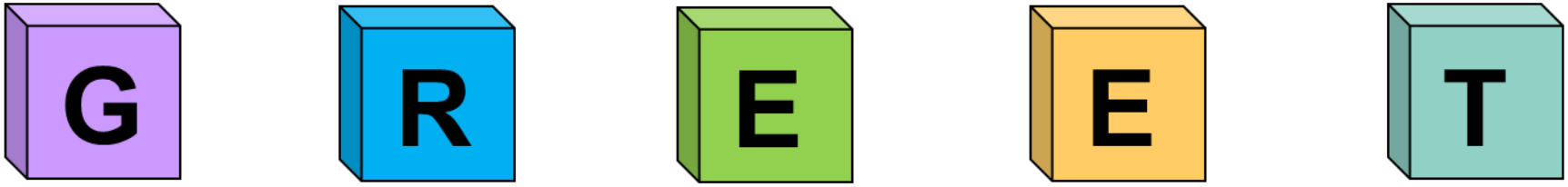
The GREET Model



Generation

How acculturated is the patient?

Reasons

Why did the patient immigrate?

Extended Family

Has acculturation been affected by family?

Ethnic Behavior

Traditional language, clothing, and rituals may be indicative of the degree of adjustment to US culture

Time living in the US

Amount of time living in the country may suggest lower acclimation

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The Campinha-Bacote Model

01 Cultural awareness
Self-examining your own biases and exploring your own cultural and professional background

02 Cultural knowledge
Seeking and obtaining sound educational base about culturally diverse groups. Integrating learned health-related beliefs and values into practice

03 Cultural skills
Collect relevant cultural data regarding a client's presenting problems and accurately conducting a culturally-based physical assessment

04 Cultural desires
Internal motivation of HCP to engage in the process of becoming culturally competent

05 Cultural encounters
Engaging in face-to-face cultural interactions with clients from culturally diverse backgrounds to modify existing beliefs and prevent stereotyping

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The Purnell Model of Cultural Competency

The Purnell Model for Cultural Competency describes primary and secondary characteristics of culture, with an additional focus on an unconscious to conscious spectrum alongside an incompetent to competent spectrum. There are also 12 aspects of this person-model that contribute to cultural competence. You can think of the Purnell Model as concentric circles of global society, community, family, and person (depicted on the next page).

Primary Characteristics	Secondary Characteristics	
• Age • Generation • Nationality • Race • Color • Gender • Religion	• Educational status • Socioeconomic status • Occupation • Military status • Political beliefs • Urban vs rural residence • Enclave identity	• Marital status • Parental status • Physical characteristics • Sexual orientation • Gender issues • Reasons for migration (sojourner, immigrant, undocumented status)

Unconsciously Incompetent	Consciously Incompetent	Consciously Competent	Unconsciously Competent
Not being aware that one is lacking knowledge about another culture	Being aware that one is lacking knowledge about another culture	Learning about the client's culture, verifying generalizations about the client's culture, and providing culturally specific interventions	Automatically providing culturally congruent care to clients of diverse cultures

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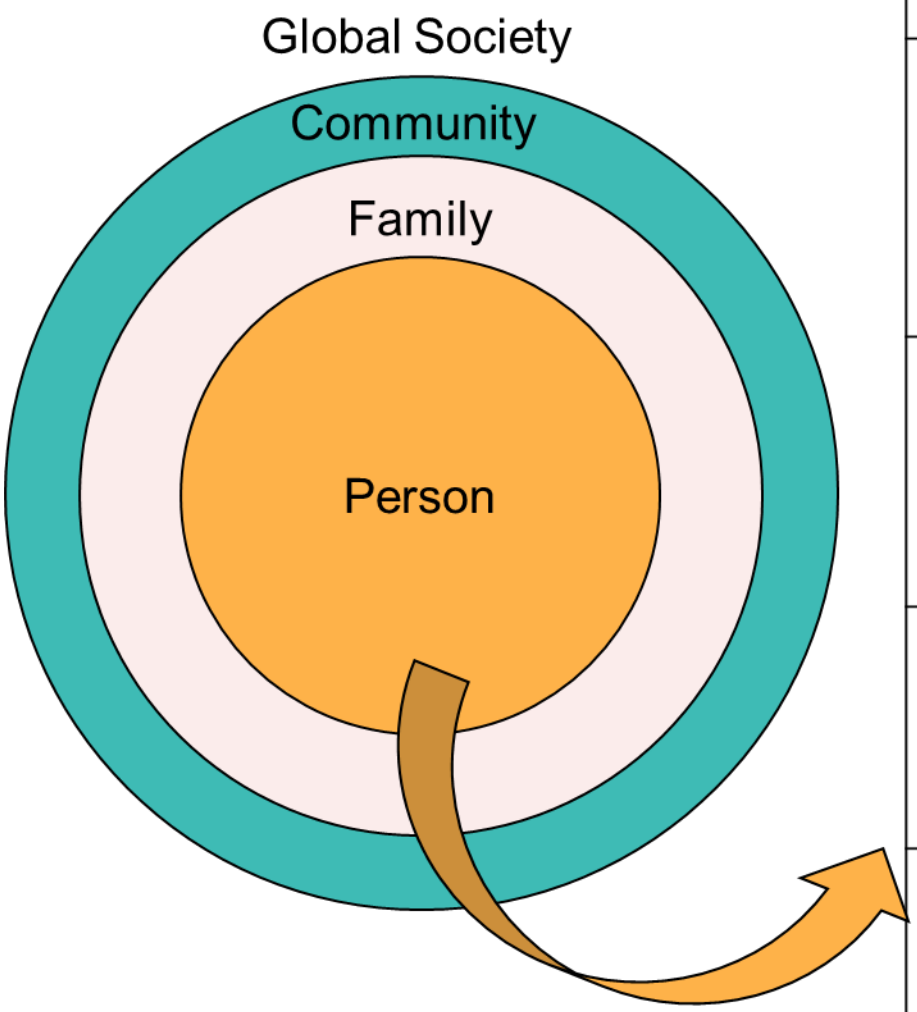

Workforce issues Acculturation Autonomy Language barriers	**Death rituals** Death rituals Bereavement	**Health-care practitioners** Perceptions of practitioners Folk practitioners Gender & health care
Pregnancy Fertility practices Views towards pregnancy Pregnancy beliefs Birthing postpartum	**Overview/heritage** Origins Residence Topography Economics Politics Education Occupation	**High-risk behaviors** Tobacco Alcohol Recreational drugs Physical activity Safety
Bicultural ecology Biological variations Skin color Heredity Genetics Ecology Drug metabolism	**Health-care practices** Focus on health care Traditional practices Magicoreligious beliefs Responsibility for health	Transplantation Rehabilitation/chronicity Self-medicine Pain/sick role Mental health barriers
Spirituality Religious practices Use of prayer Meaning of life Individual strength Spirituality & health	**Family roles and organization** Head of household Gender roles Goals & priorities Developmental tasks	Roles of aged Extended family Social status Alternative lifestyles
Nutrition Meaning of food Common foods Rituals Deficiencies Limitations Health promotion	**Communication** Dominant language Dialects Contextual use Volume/tone Spatial distancing Eye contact	Facial expressions Greetings Temporality Time Names Touch

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Practice Questions (Part 2)

Answer these practice questions using the Study Smarter Method by Veritense™ that you read about in Chapter 2 and 3 of the Study Smarter Method for the RD-Exam section. Instead of answering these questions using your memory and logic, use the study guide text and/or your own research to determine which answer is correct (and why), and why the other answers are incorrect. Pay attention to the question itself, and be sure you know what the question is asking. There are answers with explanations following. If this is your first set of practice questions, go back and skim Chapters 2 and 3 in the Study Smarter Method for the RD-Exam section to remind yourself of the types of questions and how to answer them.

1. Which of the following statement sis FALSE regarding systems theory?
    a. Organizations exist in dynamic equilibrium with the environment
    b. Organizations are a collection of parts united by interactions
    c. Organizations are a collection of simple units that should be considered separately from one another
    d. Organizations should be designed to accomplish specific objectives

2. A marketing manager asks employees to think of new ways to reach their target audience. Which of the 4 P's does this activity fall into?
    a. Price
    b. Promotion
    c. Place
    d. Product

3. _____ requires awareness and sensitivity to historic realities in addition to awareness of your own personal beliefs.
    a. Cultural competence
    b. Cultural humility
    c. Cultural sensitivity
    d. None of the above

4. Which cultural competence model focuses on overcoming communication barriers in healthcare encounters?
    a. Purnell
    b. LEARN
    c. BATHE
    d. GREET

5. Cultural _____ refer to a healthcare providers internal motivation to engage in the process of becoming culturally competence.
    a. Skills
    b. Awareness
    c. Desires
    d. Knowledge

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6. Which cultural competence model was developed to address the effect of acculturation on a patient's health?
    a. FOCUS
    b. GREET
    c. BATHE
    d. LEARN

7. The BATHE model stands for:
    a. Background, affect, trouble, handling, empathy
    b. Background, acknowledge, trouble, handling, explain
    c. Beliefs, affect, treatment, healers, explanation
    d. Beliefs, affect, time living in US, healers, ethnic behavior

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Practice Questions (Part 2) (Answers)

1. Which of the following statement sis FALSE regarding systems theory?
    a. Organizations exist in dynamic equilibrium with the environment
    b. Organizations are a collection of parts united by interactions
    c. Organizations are a collection of simple units that should be considered separately from one another
    d. Organizations should be designed to accomplish specific objectives

Answer: C. Organizations are highly complex, and they cannot be understood by considering isolated parts, because the behavior of the total organization system depends on the interaction between parts. Review the Six Foundational Concepts of System Theory.

2. A marketing manager asks employees to think of new ways to reach their target audience. Which of the 4 P's does this activity fall into?
    a. Price
    b. Promotion
    c. Place
    d. Product

Answer: B. Promotion includes marketing communication strategies and techniques such as how the company is reaching their target audience and how their product is perceived by their end user. Promotion is the communication part of the entire marketing function. **Price** affects how a product is sold and is linked to perceived value of product. **Place** refers to how the product is provided to the customer. Placement strategy assesses the most suitable channel and must complement the rest of the product strategy. **Product** is the tangible good or intangible service developed to meet a specific consumer need and demand.

3. _____ requires awareness and sensitivity to historic realities in addition to awareness of your own personal beliefs.
    a. Cultural competence
    b. Cultural humility
    c. Cultural sensitivity
    d. None of the above

Answer: B. Cultural humility is the ability to maintain an interpersonal stance that is "other-oriented" (open to the other) in relation to aspects of cultural identity most important to the client. Cultural humility requires awareness and sensitivity to historic realities (including legacies of violence and oppression against certain groups) in addition to exploring and awareness of your own personal beliefs. Please read this article on cultural humility in healthcare cited in the text, if you haven't already: https://bit.ly/3wjGOnx . **Cultural competence** is a set of congruent attitudes, behaviors, and policies that come together; organizations and institutions implement systems to apply cultural competence into practice. *What are the shortcomings of cultural competence?* **Cultural sensitivity** is awareness of similarities and differences between different cultures.

4. Which cultural competence model focuses on overcoming communication barriers in healthcare encounters?
    a. Purnell
    b. LEARN
    c. BATHE
    d. GREET

Answer: B. The LEARN model was designed to overcome communication barriers in health care encounters. The model stands for: **L**isten, **E**xplain, **A**cknowledge, **R**ecommend treatment, **N**egotiate agreement. The Purnell model of cultural competency is derived from multiple theories and includes 16 assumptions about culture. There are primary and secondary characteristics of culture, and then a spectrum of competence from unconsciously incompetent to unconsciously competent. The GREET model involves exploring acculturation and acclimation when working with immigrant populations to help healthcare providers understand their patient's context and how they understand their information in relation to their illness. GREET stands for **G**eneration, **R**eason, **E**xtended family, **E**thnic behavior, **T**ime living in the United States. The BATHE model helps create a culturally competent environment, focusing on the psychosocial context for the patient's visit rather than medical details. The model stands for: **B**ackground, **A**ffect, **T**rouble, **H**andling, **E**mpathy. *Did you find this question to be vague and tricky? You could argue that al the answer choices address communication barriers, but the BEST answer would be the LEARN model. When answering questions for the RD exam, you shouldn't have to argue your selection. For example, much of a healthcare providers interaction with a patient involves communication, but when you look at what the letters in each model stand for, which ones are about how the provider communicates and which are about additional information that should be obtained? If you were looking for the answer choice "ETHNIC model" you're on the right track, as that model is also communication focused*. *The ETHNIC model involves considering how a patient's understanding of their illness and treatment fit with their culture. This model focuses on questions and conversations providers should have with patients to achieve their goals. The model stands for: Explanation, Treatment, Healers, Negotiation, Intervention, Collaboration*

5. Cultural _____ refer to a healthcare providers internal motivation to engage in the process of becoming culturally competence.
    a. Skills
    b. Awareness
    c. Desires
    d. Knowledge

Answer: C. This question nis asking about the Campinha-Bacote Model of Cultural Competence, which includes cultural awareness, cultural knowledge, cultural skills, cultural desires, and cultural encounters. Be able to define each one.

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6. Which cultural competence model was developed to address the effect of acculturation on a patient's health?
    a. FOCUS
    b. GREET
    c. BATHE
    d. LEARN

Answer: B. The GREET model involves exploring acculturation and acclimation when working with immigrant populations to help healthcare providers understand their patient's context and how they understand their information in relation to their illness. GREET stands for **G**eneration, **R**eason, **E**xtended family, **E**thnic behavior, **T**ime living in the United States. The BATHE model helps create a culturally competent environment, focusing on the psychosocial context for the patient's visit rather than medical details. The model stands for: **B**ackground, **A**ffect, **T**rouble, **H**andling, **E**mpathy. The LEARN model was designed to overcome communication barriers in health care encounters. The model stands for: **L**isten, **E**xplain, **A**cknowledge, **R**ecommend treatment, **N**egotiate agreement. The FOCUS model is a performance improvement model.

7. The BATHE model stands for:
    a. Background, affect, trouble, handling, empathy
    b. Background, acknowledge, trouble, handling, explain
    c. Beliefs, affect, treatment, healers, explanation
    d. Beliefs, affect, time living in US, healers, ethnic behavior

Answer: A. The BATHE model helps create a culturally competent environment, focusing on the psychosocial context for the patient's visit rather than medical details. The model stands for: **B**ackground, **A**ffect, **T**rouble, **H**andling, **E**mpathy.

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Part 3

Performance Improvement Models

FOCUS – PDSA Model

The FOCUS model[45] is often combined with the PDSA model. As a manager, the FOCUS model helps you identify the problem and the PDSA model cycles you through the process of implementing a successful solution. The PDSA model and the PDSA cycle are the same thing. For the exam, know what each letter stands for and an idea of what happens in each step.

The FOCUS Model

F

Find the problem
- Identify a process to improve
- Use Pareto and Cause-and-Effect to select the most important problem

O

Organize a team
- Develop the team and establish objectives
- Interdisciplinary team more likely to find a creative solution
- Include those with a stake in the problem's resolution

C

Clarify the problem
- Explain current processes and develop the problem statement
- Break down the problem into sizeable chunks that can be analyzed and solved more easily

U

Understand the problem
- Recognition of sources of variation
- Identify the fundamental steps in the process that will bring the biggest improvement
- Generate as many possible solutions through structured thinking and brainstorming

S

Select a solution
- Use decision-making techniques to select the most viable option
- Think about consequences of the solution and make a go/no-go decision

There are worksheets in the Management Packet to help you do some active learning for all of the models presented.

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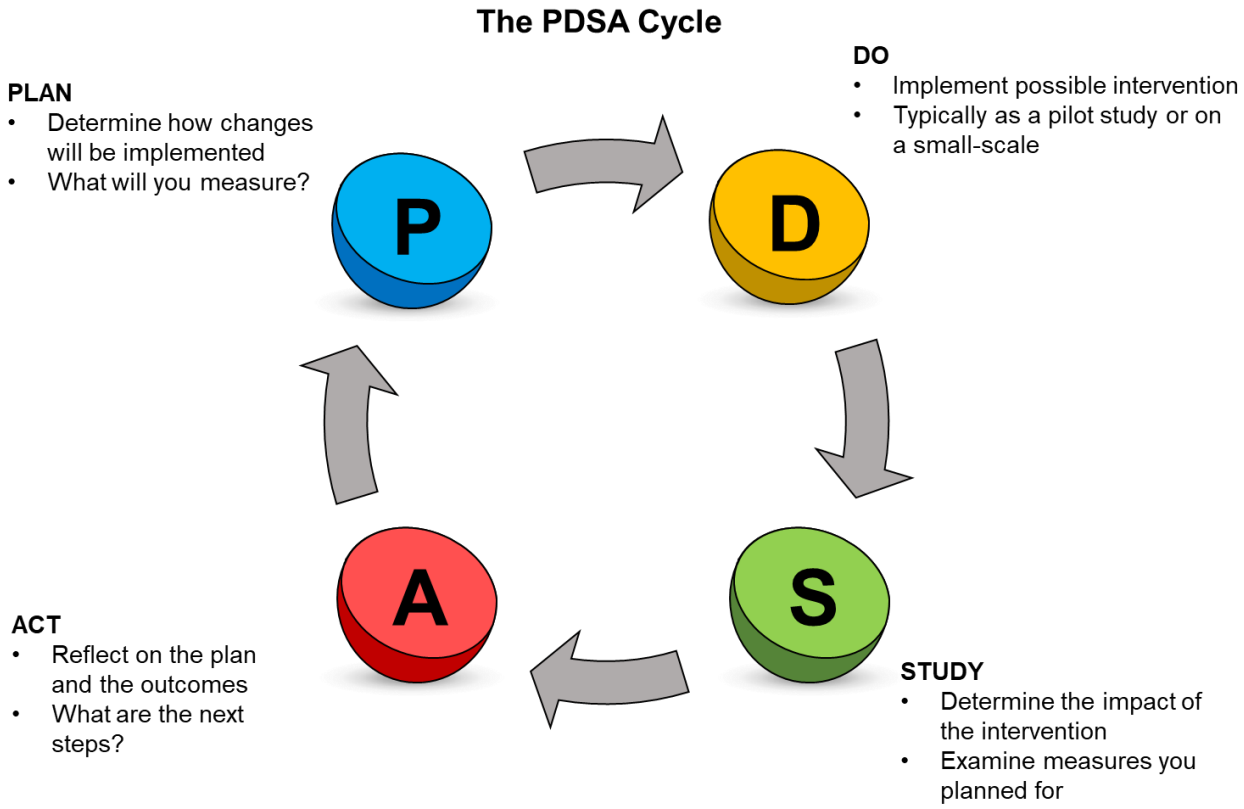
PDSA Cycle

The Plan-Do-Study-Act cycle is a four-stage, problem-solving model that is used to **improve a process or carry out change**.[45]

Remember that it is a cycle and that the team can go through several iterations for success, such as pilot studies and small-scale implementation. That's when the A cycles back to the P.



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Total Quality Management

Total Quality Management, or TQM, is focused on improving processes in response to customer needs.

TQM is a **company culture and attitude** that is driven by satisfying customers via products and services, with continuous improvement methods that involve all employees in all departments. TQM treats employees as “internal customers”.[46]

These 8 aspects are key to TQM:[46]

- Customer-focused
- Strategic approaches
- Total employee involvement
- Continuous improvement
- Process-centered
- Fact-based
- Integrated system
- Communication

Quality Improvement vs Quality Assurance

Note that there is a difference between quality improvement programs and quality assurance programs. Quality assurance “measures compliance against certain necessary standards, typically focusing on individuals”, [47] whereas quality improvement is a continuous improvement process focused on processes and systems. QA tends to be defense with a focus on providers. QI is proactive and preventative in nature, focusing on patient care.”[48] Outcomes, meeting standards, prevention, standard procedures, and diagnostics are key aspects of quality assurance programs but not quality improvement programs.

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IOM Framework for Health Care Quality

The Six Domains of Health Care Quality was put forth by the Institute of Medicine (IOM) for quality assessment. These six domains make up the "IOM Framework for Health Care Quality."

Go to http://www.ahrq.gov/talkingquality/measures/six-domains.html or type "Six Domains of Health Care Quality" or "IOM Framework for Health Care Quality" into a search engine to fill out the definitions of each domain below.

Safe	
Effective	
Patient-Centered	
Timely	
Efficient	
Equitable	

*What is the difference between **efficient** and **effective?***

*What does it mean for health care to be **equitable**?*

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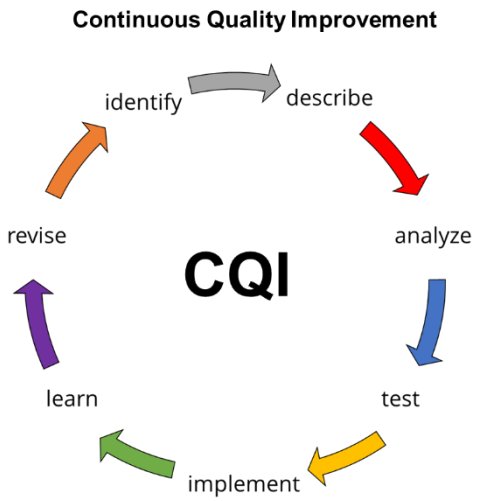
Continuous Quality Improvement



Continuous quality improvement programs:[49]

- Identify problems
- Implement corrective action
- Study the effectiveness of the solution

CQI is an ongoing cycle of collecting data and using the data to make decisions for gradual and continuous improvement to programs and processes.[49]

CQI takes a broad look at the overall system within which different programs and processes operate, and keeps that system in mind when identifying problems, implementing corrective action, and studying the effectiveness of the solution. **Does that sound like systems theory?**

The key tenants of CQI are:

- Customer satisfaction
- Data collection via the scientific method
- Team involvement

Customer satisfaction directs the CQI process towards what should be examined. Data collection, via the scientific method, provides high quality data to guide the improvement process. Team involvement encompasses the system level and includes people from all areas that the improved program or process might touch.

A non-nutrition example of CQI is how the motor vehicle industry develops new versions of cars each year, incorporating improvements based on user experience studies (such as adding technology and comfort), environmental concerns (hybrid and electric cars), and changing demographics (increased market for family-size vehicles). Look at the evolution of the Ford Mustang!

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Kaizen

Kaizen is a Japanese word for “continuous improvement” or “change for the better”. “Kai” means change and “zen” means “for the better”. The concept of KAIZEN™ is a Japanese business philosophy regarding the processes that continuously improve operations and involve all employees.[50] It was introduced in 1986 by Masaaki Imai.

KAIZEN™ can be applied to personal or work life and the main differentiating factor of this continuous improvement method compared to others, in the work space, is the involvement of *all* employees, rather than dictating changes from management levels down to workers. This process can be used to improve product quality, worker productivity, and relationships between labor and management.

The 5 Principles of KAIZEN™ are:

1. Know your customer – create customer value by identifying their interests and then enhance the customer experience.
2. Let it flow – everyone in the organization should aim to create value and eliminate waste
3. Go to Gemba – value is created where things actually happen, and so you should follow the action. Gemba refers to *workplace,* with the dichotomy that many managers work in a separate area from where laborers work, such as in a manufacturing or packaging facility. How can managers make effective changes if they never observe and participate in how things are actually done?
4. Empower people – set the same goals for teams and provide a system and tools so they can reach their goals.
5. Be transparent – use real data to show tangible and visible performance improvement results

The KAIZEN™ process involves practicing, learning, and improving, and adhering to the 5 principles with determination. The Kaizen Institute describes the process in these clear statements:[51]

- Good processes deliver good results
- Go see for yourself to grasp the current situation
- Speak with data, manage by facts
- Take action to correct root causes of problems
- Work as a team
- KAIZEN™ is everybody’s responsibility

The KAIZEN™ Change Model has 4 pillars:

1. Daily KAIZEN™ - change Gemba behaviors and culture
2. Breakthrough KAIZEN™ – to implement new paradigms and processes
3. Leaders KAIZEN™ – to develop KAIZEN leaders
4. Support KAZIEN™ – build long term capability with steering and infrastructure to embed KAIZEN™ Culture

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The Kaizen Institute has a Youtube channel – now that you've been exposed to the information for the first time, help it sink in further by watching some of their informational videos. You can also read through some KAIZEN™ case studies at www.kaizen.com/blog to take these abstract principles and pillars and see how they are applied in the real world.

FADE Quality Improvement Model

The FADE Quality Improvement Model, sometimes called the FADE Performance Model, was established to improve healthcare quality.

The FADE model incorporates 4 key aspects of effective quality improvement initiatives:[52]

1) Considers both systems and processes
2) Focuses on patients
3) Focuses on being part of the team
4) Focuses on the use of data

You can see that with aspects 1, 3, and 4, the FADE model is similar to the CQI model, which makes sense because they are both focused on quality improvement.

FADE stands for Focus, Analyze, Develop, and Execute.

FADE Quality Improvement Model

Focus	Analyze	Develop	Execute
Generate a list of problems Select 1 problem Verify and define the problem	Decide what you need to know Collect data baselines and patterns Determine influential factors	Generate promising solution Select solution Develop implementation plan	Gain commitment Execute plan Monitor impact

We **focus** by generating a list of problems, selecting 1 problem, and then verifying and defining the problem. We **analyze** by deciding what we need to know, collecting data baselines and patterns, and determining influential factors. We **develop** by generating promising solutions, selecting solutions, and developing implementation plans. Finally, we **execute** by gaining commitment from the team, executing the plan, and monitoring impact.

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An important part of nearly all of the models presented in this section is that they take time at the beginning to assess what really needs to be done, and they take time at the end to see if it's working. We don't assume we know what's best to fix a situation, go in and do it, and then move on to the next part.

RUMBA

The RUMBA model stands for reasonable, understandable, measurable, believable, and achievable.[53] It's an individualized model that you can use as a manager to engage your team with change. *An effective leader accepts change as a part of life and works with their team to embrace it.*

RUMBA

R	**Reasonable** Creating reasonable goals increases team buy-in and creative processes
U	**Understandable** Verbalizing team goals in an easy-to-understand way means everyone is on the same page
M	**Measurable** By creating measurable goals, the team won't become disillusioned
B	**Believable** Demonstrate your belief (as manager) in the success of the project
A	**Achievable** Explaining specific milestones is key (in addition to laying out the overall plan) to stimulate personal achievement

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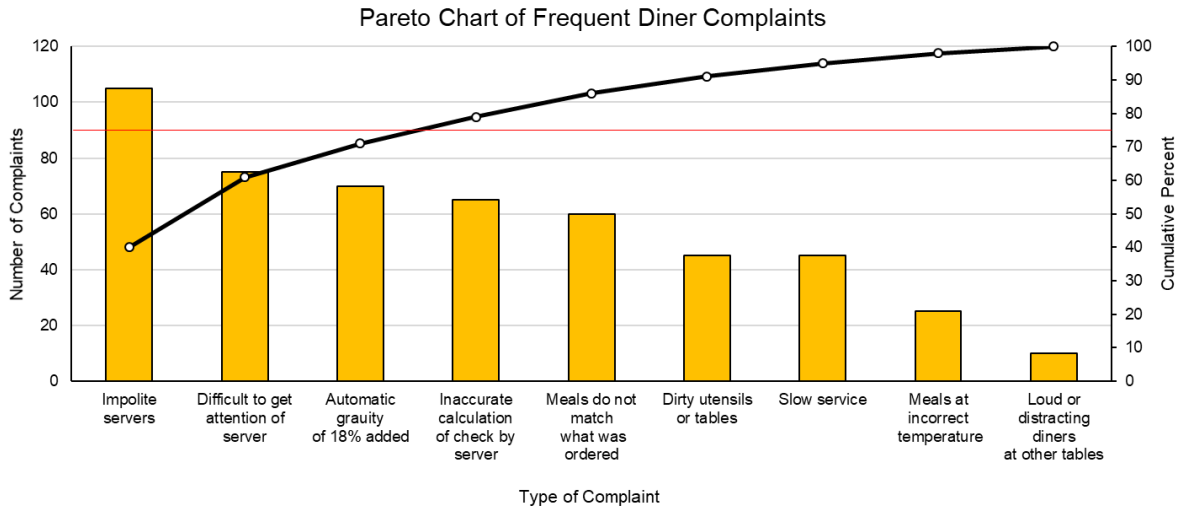
Project Management Tools

Pareto Chart

In the FOCUS model section, the diagram mentions Pareto Charts and cause and effect charts in “F” for “Find the problem”. Pareto charts have 2 components: a bar graph that represents the descending frequency of occurrence (or cost), and a line graph representing the cumulative percentage of total occurrences (or cost).

Why do we use Pareto charts? By ordering the occurrence of a complaint in descending order, we can identify the complaint that has the largest effect on satisfaction.[35] We can also determine the number of factors we need to address in order to reduce complaint frequency by a certain percent. If the chart represents cost rather than occurrence, then you can imagine that addressing the most expensive problem, or knowing how many problems you need to address to cut costs to meet your budget, would be helpful.

Example: Diner Complaints - The chart tabulates the number of diner complaints in categories: impolite server, difficulty getting server’s attention, automatic gratuity of 18%, inaccurate check calculations, incorrect meals, dirty utensils or tables, slow service, serving cold meals (incorrect temperature), and loud or distracting diners at other tables. On the secondary axis, we have the cumulative percent of each complaint. We use the cumulative percent to figure out that if we want to reduce complaints by 75% (horizontal line), we should address the top 3 factors – impolite servicers, difficulty getting servers attention, and automatic 18% gratuity, because if we follow the thick line corresponding to the secondary access, we reach about 70% after the first 3 categories. If we didn’t construct this Pareto chart, we might instead focus on other factors on the list based on our subjective improvement, and then not see the improvement in complaints we are looking for. It’s all about collecting data to fix your problem.



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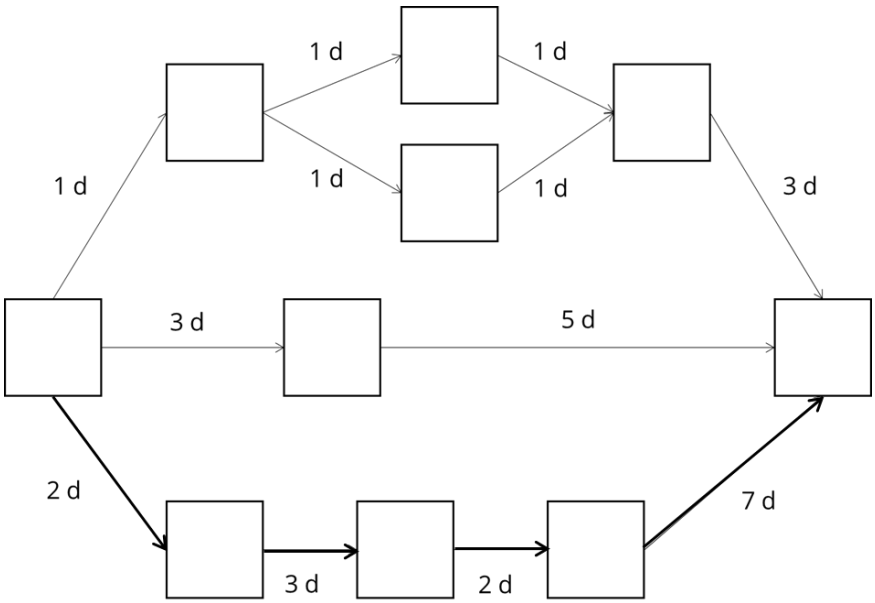
PERT Chart

PERT chart stands for “Program Evaluation Review Technique” and is a project management **evaluation tool** used for effective planning at the management level. We use it to schedule, organize, and coordinate tasks within a project. [35]

To construct a PERT chart:

1) Each task necessary to complete a project is written in nodes (blank boxes in this example).
2) Directional arrows represent sequential tasks – one must be completed before another. Not all tasks are sequential; some can be performed at the same time.
3) The amount of time needed to complete each activity is written along each line – it could be days, weeks, hours, or months.
4) Critical path analysis is a bolded line that shows the longest path by duration to complete the project as described in the chart.



Nodes are key tasks or milestones, and might be a rectangle or a circle, with the task written inside. Or, the node might have a number and the tasks listed in a separate table. Parallel tasks without direct arrows going from one task to another can occur at the same time. Breaking a project down into necessary tasks is a key step in effective project management in general. Creating sequences between the tasks is helpful, especially if different people are working on those tasks. Critical path analysis provides you, as the manager, and your boss, with an idea of the longest time the process might take.

PERT charts may also be used to work backwards from a hard deadline. We would subtract the amount of time needed for each activity to get our start date.

Finally, PERT charts are helpful for project managers to evaluate and analyze which resources (labor, time, materials) are needed for a project, as well as track the progress of the project. If a project includes multiple departments, having a master PERT chart can help facilitate

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communication and make sure everyone is on the same page, especially if a task in your department is dependent on a task in another department.

The time component of the PERT chart is invaluable. As a manager, you can estimate the shortest time the project might take, the longest, and “what-if” analyses, like “what if step 2 takes longer than we expect?” Any step or task that has a large impact on the final deadline is a “risk” and should be carefully monitored.

Note that the time estimates for PERT charts are just that – estimates. It’s a planning procedure and is better than not considering time at all, but the time requirement isn’t guaranteed. PERT charts work best if you have detailed knowledge of the tasks and processes that will be involved and can easily fill out the components. If a large amount of research is required to determine duration and cost, and the time spent on that research isn’t offset by the benefit of the PERT chart, then another project planning tool may be a better choice.

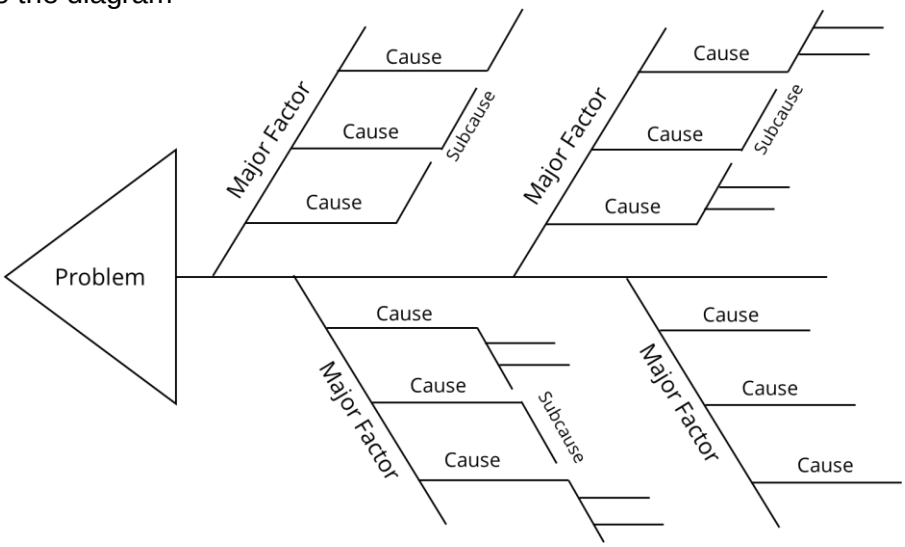
Cause and Effect Diagram

Cause and effect diagrams are also called “Ishikawa” diagrams, named after their developer, Kaoru Ishikawa, or “Fishbone Diagrams’ because of the format.

These diagrams can be used to identify problems for continuous quality improvement programs, like the F step of the FOCUS model.[54] It can also be used in less formal processes to discover bottlenecks and root causes of problems.

To construct a cause and effect diagram:

1) Write down the problem to one side as the fish head
2) Brainstorm major factors that might affect the situation by drawing lines off the center spine
3) Write down causes (horizontal lines) that contribute to each major factor
4) Add further sub-causes with more branches off of the cause lines (vertical) and their factors (horizontal)
5) Analyze the diagram



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An advantage of cause-and-effect diagrams is that they often reveal causes that are not obvious at first, and if addressed, lead to effective solutions.

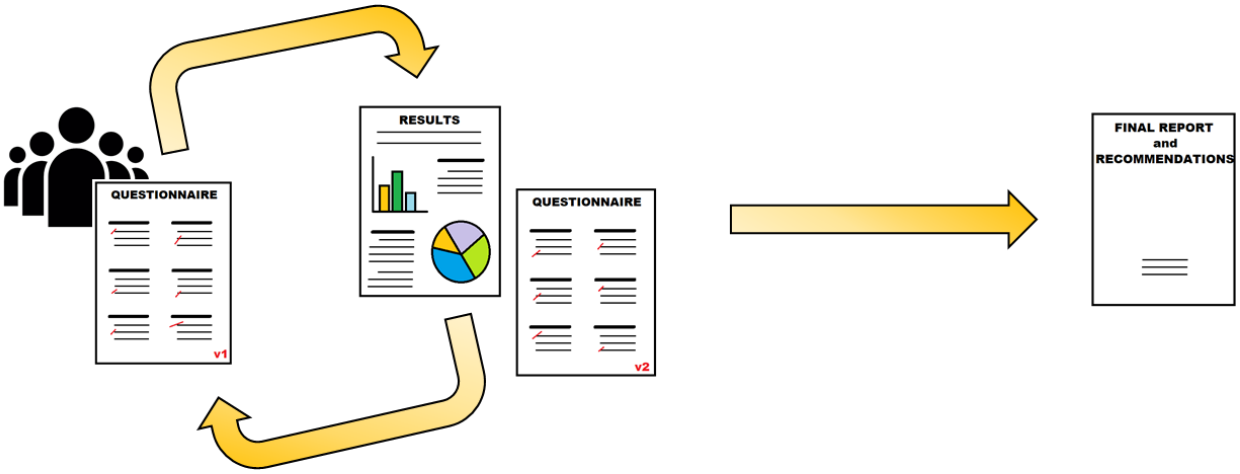
What would the diagram look like if you only had major factors and causes (no sub causes or sub-factors)? What if you had sub-causes and factors for each node?

Planning Processes

There are several planning processes that may be used in keyword questions on the exam. These include:

- Delphi method
- Delbecq's nominal group technique
- Charettes

Delphi method: a group of experts are contacted individually and anonymously with questionnaires.[55] Their responses are statistically tallied and summarized in a report which is sent back to the experts with another questionnaire. The goal of the Delphi method is to reduce the range of responses from experts and arrive at what might be called a consensus.



Delbecq's nominal group technique: using structured, small-group discussion to reach a consensus.[56] The moderator presents the question of interest. Each group member writes down their ideas separately. Then each person shares their idea and the moderator writes them on a group board. The group then discusses each idea, revising them, and then votes on the best one.

Charrette: collaborative planning process harnessing the talents of all interested parties.[57] Results in a master plan that represents transformative community change.

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Lean Six Sigma

As you read through this section, jot down your own study guide questions. What information are main takeaways? Turn that answer around into a question.

Lean

"Lean" is a "systematic approach to eliminate activities that don't add value to a process"[58] with the goal of high quality and customer satisfaction. Waste removal is a continuous process. To use Lean, you have to define value – or what the customer's perception of the product or service is and what they are willing to pay for. You can then label work activities as value-added or non-value-added.

Lean evolved from the Toyota Production System and uses the 3M model of Muda, Mura, and Muri to expose inefficient processes.[58]

> **Muda** is any activity that does not add value to a product or service, and is the Japanese word for waste. There are 8 types of Muda: defects, overproduction, waiting, non-utilized talents, transportation, inventory, motion, extra processing → it spells DOWNTIME.
>
> **Mura** is the concept of inconsistencies in business operations that results in production fluctuations, and thus suboptimal resource utilization. Uneven workloads, inventory accumulation, and waiting are examples of Muda that are Mura. These are particularly applicable to a foodservice setting. What foodservice system type has an uneven distribution of food preparation work? What measures do we use to quantify the value of products in inventory?
>
> **Muri** is the occurrence of dissatisfaction amongst people or unused resources, such as machinery. Customers left waiting, ovens not being utilized, and staff being over stressed are examples of Muri. Muri leads to Muda.

The 5 Principles of Lean are: 1) Identify your customers and what they value; 2) map the value stream; 3) create flow to the customer; 4) establish pull based on customer demand, and 5) seek continuous improvement.[59]

Lean approaches use tools like the PDSA model, 5S (Sort, Set in Order, Shine, Standardize, Sustain), and 8 wastes.

Six Sigma

The goal of Six Sigma is to create effective processes. Six Sigma itself is a problem-solving methodology that uses *data* and the DMAIC framework – define, measure, analyze, improve, and control. [58] If you've read through the previous sections in this study guide then this framework should look familiar.

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- **Define** – Define the problem and objectives. This may be done with a problem statement or goal statement, as well as listing out the scope of the project, necessary resources, anticipated benefits, and the project timeline.
- **Measure** – What do we need to improve, and can we measure it? Identify and measure baseline metrics to compare to final performance later.
- **Analyze** – Analyze the process and define the factors of influence. Conduct "root cause analysis" with Pareto charts and fishbone diagrams. Verify and validate root causes with statistical analyses.
- **Improve** – Identify and implement improvements. Form solutions and test them. Do they eliminate the root causes? In this stage, you may run simulation studies or pilot interventions.
- **Control** – Ensure that improvements will be sustained. Record the performance of the solutions implemented in the Improve step, and put a control system in place to monitor performance. Standardized processes using instructions and verify estimated project benefits against anticipated benefits.

Lean Six Sigma

Lean Six Sigma combines the waste-reduction goals of Lean with the problem-solving methodology of Six Sigma. First steps may use Lean to make a workplace efficient and effective, using less technical tools like KAIZEN and workplace organization, followed by Six Sigma statistical analyses to identify root causes for any problems that remain.[60]

You can think of Lean Six Sigma in terms of 3 elements: customers, processes, and employees.[58] Customers are anyone who is consuming a business' products or services. Processes are core to Lean, with inefficient processes leading to poor quality and profit losses. Finally, similar to how KAIZEN™ emphasizes involving managers and workers in changes, successful Lean Six Sigma initiatives or infrastructure changes require everyone in the organization to be involved and invested.

Now that you've read through this material, go online and search for videos on Lean Six Sigma to help learn the information in a different way.

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Practice Questions (Part 3)

Answer these practice questions using the Study Smarter Method by Veritense™ that you read about in Chapter 2 and 3 of the Study Smarter Method for the RD-Exam section. Instead of answering these questions using your memory and logic, use the study guide text and/or your own research to determine which answer is correct (and why), and why the other answers are incorrect. Pay attention to the question itself, and be sure you know what the question is asking. There are answers with explanations following. If this is your first set of practice questions, go back and skim Chapters 2 and 3 in the Study Smarter Method for the RD-Exam section to remind yourself of the types of questions and how to answer them.

1. Which of the following statements is TRUE regarding quality assurance (QA) programs?
    a. QA is patient-centered and focuses on preventing problems
    b. QA typically involves meeting established standards and individuals' compliance with standard procedures
    c. QA is a continuous process that focuses on the organization's systems
    d. QA is dedicated to identifying problems and then studying the effectiveness of the solution

2. Which performance improvement model is unique in its involvement of all employees in making changes to the workplace environment?
    a. FADE
    b. CQI
    c. PDSA
    d. KAIZEN™

Write your own questions for the concepts of Total Quality Management, Continuous Quality Improvement, the FADE model, the RUMBA model, and the IOM Framework for Health Care Quality.

3. Which project management tool shows the frequency of costs with the cumulative percentage of total costs?
    a. Pareto chart
    b. PERT chart
    c. Cause and effect diagram
    d. Fishbone diagram

4. Which project management approach uses the 3M model?
    a. Lean Six Sigma
    b. KAIZEN™
    c. Total Quality Improvement
    d. Continuous Quality Improvement

5. Which of the following is not a pillar of the KAIZEN™ Change Model?
    a. Daily KAIZEN
    b. Breakthrough KAIZEN
    c. Review KAIZEN
    d. Support KAIZEN

6. Which of the following is not a principle of KAIZEN™?
    a. Know your customer
    b. Be the change
    c. Empower people
    d. Go to gemba

7. What does *gemba* mean?
    a. Change
    b. For the better
    c. Workplace
    d. Inspiration

Practice Questions (Part 3) (Answers)

1. Which of the following statements is TRUE regarding quality assurance (QA) programs?
    a. QA is patient-centered and focuses on preventing problems
    b. QA typically involves meeting established standards and individuals' compliance with standard procedures
    c. QA is a continuous process that focuses on the organization's systems
    d. QA is dedicated to identifying problems and then studying the effectiveness of the solution

Answer: B. quality assurance (QA) programs are typically individual-focused, rather than systems- or process-focused, and involves measuring compliance against standards. You can think about QA in contrast to quality improvement (QI). QA is often defensive or reactive, while QI is proactive and continuous, focused on exploring the problem and potential solutions. QA is focused on determining if employees or teams are adhering to standardized procedures and meeting benchmarks.

2. Which performance improvement model is unique in its involvement of all employees in making changes to the workplace environment?
    a. FADE
    b. CQI
    c. PDSA
    d. KAIZEN™

Answer: D. The concept of KAIZEN™ is a Japanese business philosophy regarding the processes that continuously improve operations and involve all employees. TQM, or total quality management, is a company culture/attitude that also emphasize involving all employees in all departments. The FADE model incorporates 4 key aspects of effective quality improvement initiatives: Considers both systems and processes, focuses on patients, focuses on being part of the team, and Focuses on the use of data. FADE stands for Focus, Analyze, Develop, Execute. CQI, or Continuous Quality Improvement, is an

ongoing cycle of collecting data and using the data to make decisions for gradual and continuous improvement to programs and processes. Kaizen is a type of CQI method. The PDSA cycle is a 4-stage problem-solving model used to carry out change to improve a process and stands for plan, do, study, act.

3. Which project management tool shows the frequency of costs with the cumulative percentage of total costs?
    a. Pareto chart
    b. PERT chart
    c. Cause and effect diagram
    d. Fishbone diagram

Answer: A. The Pareto chart involves a bar graph of occurrences or costs in descending order for one y-axis and the cumulative percentage of total costs in line graph form using the other y-axis. A PERT chart is a project management evaluation tool used for effective planning by estimating the amount of time needed to complete each task and determining the critical path. A cause-and-effect diagram, or a fishbone diagram, involves writing down the problem and then breaking down major factors into causes and subcauses.

4. Which project management approach uses the 3M model?
    a. Lean Six Sigma
    b. KAIZEN™
    c. Total Quality Improvement
    d. Continuous Quality Improvement

Answer: A. Lean Six Sigma combines the waste-reduction goals of Lean with the problem-solving methods of Six Sigma. The goal of lean is to eliminate activities that don't add value to a process, and uses the 3M model of Muda (8 types of waste), Mura, and Muri. *What are Muda, Mura, and Muri? What are the 5 principles of Lean*? *What tools are used in Lean?* The goal of Six Sigma is to create effective processes and uses the DMAIC framework (what does DMAIC stand for?) First steps may use Lean to make a workplace efficient and effective, using less technical tools like KAIZEN and workplace organization, followed by Six Sigma statistical analyses to identify root causes for any problems that remain. The concept of KAIZEN™ is a Japanese business philosophy regarding the processes that continuously improve operations and involve all employees. TQM, or total quality management, is a company culture/attitude that also emphasize involving all employees in all departments. CQI, or Continuous Quality Improvement, is an ongoing cycle of collecting data and using the data to make decisions for gradual and continuous improvement to programs and processes.

5. Which of the following is not a pillar of the KAIZEN™ Change Model?
    a. Daily KAIZEN
    b. Breakthrough KAIZEN
    c. Review KAIZEN
    d. Support KAIZEN

Answer: C. The 4 pillars of the KAIZEN Change Model are 1) daily KAIZEN™, 2) breakthrough KAIZEN™, 3) leaders KAIZEN™, and 4) support KAIZEN™.

6. Which of the following is not a principle of KAIZEN™?
    a. Know your customer
    b. Be the change
    c. Empower people
    d. Go to gemba

Answer: B. The idea of "be the change" is in line with the KAIZEN Guiding Principles – take action to correct root causes of problems and KAIZEN is everybody's responsibility, but is not a principle in itself. The 5 principles of KAIZEN™ are: 1) know your customer, 2) let it flow, 3) go to gemba, 4) empower people, 5) be transparent.

7. What does *gemba* mean?
    a. Change
    b. For the better
    c. Workplace
    d. Inspiration

Answer: C. Gemba refers to *workplace* and "go to gemba" is a principle of the continuous improvement model, KAIZEN™. "Go to Gemba" encourages managers to go to where the action is happening so that they can observe and experience the process that they are suggesting changes for.

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